California Orthopaedic Association

Sample Medical-Legal Report

This document is presented as an example of the form and general content of an orthopaedic QME/AME report. The specific diagnoses, conclusions, and opinions expressed herein are those of the author and not necessarily those of the California Orthopaedic Association or every California Orthopedist.

(Put on AME's Letterhead)

(Date of Evaluation)

Department of Industrial Relations
Division of Worker's Compensation
Disability Evaluation Unit
(Address)

Re:	Tom Patient
SSN:	(Social Security Number)
D/I:	(Date of Injury)
C/N:	(Ins. Co. Claim #)
WCAB:	(WCAB Claim #)
Imp:	(Name of Employer)
D/Exam:	(Date of Exam)
D/Dict:	(Date Report Dictated)

Agreed Medical Evaluation – Orthopaedic Surgery

The above-captioned patient was seen in this office on January 27, 2005 for an orthopaedic evaluation with the undersigned acting in the capacity of an Agreed Medical Examiner.

This examination was assisted by the presence of a professional Spanish interpreter, (Name & Certificate #).

The patient is a 33-year-old, right-handed, Hispanic male, who worked in a factory that made kitchen cabinets. His job was to paint the cabinets. He states he would have to lift cabinets and he would have to lift up to 200 pounds with assistance. This was a full-time job. He started this job in June or July 2004 and had no concurrent employment.

The patient gives the following history. (In spite of the presence of an interpreter, it is very difficult to get a good history from the patient who changes his history.)

The patient reports that on November 30, 2004 he was lifting a cabinet with a co-worker and the cabinet started to fall and he held onto it and felt a pulling in his lower back. He completed the remaining five hours of his work shift. He says the pain was not very severe initially.

He remained at work for the next two to three days. The pain was getting worse and he reported it. He was sent to a chiropractor at the end of his work shift. He says he was only examined and given no treatment. Initially he states he was not taken off work but later, in trying to find out when he stopped working, he says he was told not to work. He has never returned to work since then.

He says that rather than getting treatment in the office of that doctor he was advised just to apply ice or heat.

At the second evaluation in that office, he was also given no treatment.

Following that, the company sent him to a medical doctor whose name he cannot recall. He says x-rays were taken. He was given medication and started on physical therapy with electrical stimulation and aquatherapy three times weekly. He had the treatment for about two to three months. When this treatment ended, he was still feeling the same. He says he was then released from care and told to go back to work.

The patient did not go back to work. He called in and was told he would be called back but he was never called back.

He then got an attorney. The attorney sent him to Dr. B. He was prescribed a back support, a TENS unit for home use, and medication. Later he was started on physical therapy, going three times weekly for electrical stimulation and acupuncture. Exercises were given but they bothered his back. He is still getting physical therapy and in spite of this prolonged treatment he still feels the same.

Additionally he says he was sent by Dr. B to Dr. F who gave him injections twice a week for the neck and back. (These sound almost like epidural injections, but getting epidural injections twice a week for quite some time does not sound reasonable.) He indicates he cannot estimate how many of these injections he had, but he says he did have a lot. He says with the injections he only got relief that lasted for three days.

He has had diagnostic studies that sounds like an EMG and MRI study, but he does not know the results of these studies.

Present Complaints

The patient reports headaches. He says that since this injury he has to wear his glasses more frequently than previously. (Please note: a straining injury of the lower back should not have any effect on the need for use of glasses.)

He has neck pain. He indicates the neck pain first started approximately one and one half months after this injury. Initially it was only a very mild pain. He says he did tell the first doctor about the neck pain but the neck was not treated.

With reference to the neck he states there is a constant pain that is aggravated by looking up or down or lying on a pillow. The pain from the neck radiates into the lower back. The pain from the neck also radiates into both upper extremities to the fingers of both hands, more strongly in the left than the right. He says the pain going into his hands started when his neck pain got worse. He says the neck pain is stronger than the arm pain. Additionally there is tingling in both hands.

He reports low back pain that is a constant pain. The lower back pain is stronger than the neck pain. The lower back pain is aggravated by lying down, by standing and by sitting. (Please note: He did frequently change from standing to sitting while giving his history.)

He says that for relief of his back pain he takes medication and a change of position helps.

The lower back pain radiates to the toes of both feet, more strongly on the left than the right. The back pain is stronger than the leg pain. There is numbness and tingling in both legs. The back pain is aggravated by coughing and sneezing.

Previous Injury History

The patient reports no history of a previous injury involving his neck or back, and no previous injury of any kind whatsoever. He reports no subsequent injuries.

He states this was his only lifetime injury.

Personal History

The patient has a common-law living arrangement with one child. Tobacco: None. Alcohol: None. Medications: The patient is taking Tylenol, Ranitidine, Cardosoprol, and Naprosyn. Allergies: None.

Past Medical History

The patient has had the usual childhood diseases. Serious Illness: The patient states he has an ulcer history. Surgeries: None. Hospitalizations: None.

Physical Examination

The patient is a well-developed, well-nourished, Hispanic male. He comes to the evaluation utilizing a cane which he says he was given by Dr. B for the past month. He carries it in his left hand, but it was not used during the examination.

Generally throughout the examination the patient reports pain with almost every movement, and following performing range of motion studies he even says he feels "dizzy."

He gets on and off the examination table slowly. He walks very slowly. He says he cannot walk on his toes or heels because of pain. He does have a normal stance and habitus. He performs only 20% of a full squat with a complaint of back pain.

Vital Signs: Blood Pressure 110/90; pulse 80. Height 5 feet 8 inches. Weight 180 pounds. Right handed.

Examination of the Cervical and Dorsal Spine:

Inspection: The shoulders are level. The spine is straight. There is no deformity evident. There is a large hyperpigmented birthmark on the lower thoracic area.

Palpation: There is no muscle spasm. There is no localized tenderness. There is no increase in local warmth.

Motions (R/L): The patient flexes his chin to his chest. Extension is 60 degrees. Neck rotation is 60/40 degrees. Neck lateral bending is 30/30 degrees. (The patient complains of pain with all movement, and with rotation the pain is greater when he turns toward the left than the right.) Foraminal closure testing produces neck pain. Cervical compression produces marked low back pain but this is even prior to the application of any pressure whatsoever. Cervical traction similarly produces a complaint of pain in the neck area but before any pressure is applied that would produce traction on the neck. Shoulder depression testing is negative.

Motor Testing: Motor function in both upper extremities is intact.

Sensory Testing: The patient reports that he has relative diminution of sensation on the left side of his face (not including the forehead). There is also circumferential diminution of sensation in the left upper extremity that extends from the elbow to the fingertips. (This sensory distribution does not follow any anatomical neurological pattern.)

Reflexes (R/L): The biceps, triceps and brachioradialis reflexes are present and symmetrical.

Measurements (R/L = inches):

Circumference of upper arm	11	10-3/4
Circumference of forearm	10-1/2	10-1/4

Jamar Dynamometer Grip Strength Testing (R/L): 100/85, 85/80, 90/75.

Examination of the Lumbosacral Spine:

Inspection: The pelvis is level. The spine is straight. No deformity is evident.

Palpation: There is no muscle spasm. There is no localized tenderness including examination by punch percussion. There is no increase in local warmth.

Motions (R/L): The patient makes no attempt to forward flex his spine because of complaint of pain. Extension is limited to 5 degrees because of pain. Lateral bending is 10/10 degrees. Trunk rotation is 30/30 degrees. (He indicates that lateral bending and rotation are also limited by pain.)

Supine leg raising by the patient at 40 degrees on the right and 30 degrees on the left with complaint of low back pain, but not sciatic pain. Fabere test is negative. Fajersztajn test is negative bilaterally. The reverse Fajersztajn test is positive on the right. (That is, at the limit of supine leg raising, dorsiflexion of the foot does not increase his pain, but plantar flexion of the right foot does increase his back pain. This is a maneuver which should relieve any stretch irritability and is a non-organic finding.)

The flip test (seated leg raising) is resisted by the patient at 5 degrees short of full extension of the knees, although he does not arch his back. The Cram test is negative. The patella shift test is negative.

Motor Testing: Motor function in both lower extremities is intact. (He complains of pain with resisted flexion and extension of the feet and toes while he lies supine.)

Sensory Testing: The patient reports relative diminution of sensation in the posteromedial left thigh and the medial aspect of the left calf.

Reflexes (R/L): Knee jerks 1+/1+; ankle jerks 0+/0+. There is no ankle clonus.

Measurements (R/L = Inches):

Leg length		37-1/2		37-1/2
Circumference of thigh	18		18	
Circumference of calf		14		14-1/4

X-Ray Report

X-rays of the cervical, thoracic and lumbosacral spine, and pelvis were obtained in this office January 27, 2005 by Certified Radiology Technician C (Certificate #), and interpreted by the undersigned.

Cervical Spine (7 Views): There is no evidence of fracture, dislocation of any other bony injury. Disc and joint spaces are well maintained. There is no arthritic spurring. The neural formina are patent.

Thoracic Spine (3 Views): There is a slight right thoracic scoliosis curve. There is no evidence of fracture, dislocation, or other bony injury. Disc and joint spaces are well maintained. There is no arthritic spurring.

Lumbosacral Spine (5 Views): There are six lumbar-type vertebrae. (This is an anatomical variant.) There is no evidence of fracture, dislocation, or other bony injury. Disc and joints spaces are well maintained. There is no arthritic spurring. There is no spondylolysis. There is no spondylolisthesis.

Pelvis (1 View): There is no bony abnormality of the pelvis. The hip and sacroiliac joints are well maintained.

Review of Records

Some records have been submitted for review. These will be summarized in chronological sequence for clarity of review.

There is a Doctor's First Report of Work Injury prepared by Dr. B. who saw the patient on December 3, 2004. (I am not sure of the exact date because the report is poorly typed.) The date of injury is listed as November 30, 2004, and it was stated he had low back pain after trying to remove a heavy cabinet from a wall. The history is of neck and low back pain since November 30, 2004 after trying to remove a heavy cabinet from a wall that weighed about 100 pounds. It was noted he had help. The examination revealed slight kyphosis. He could reach to the floor but it was painful to recover. He had full rotation. Flexion to either side was limited to 40% of normal. It was stated he had an antalgic gait. He could heel and toe walk. X-rays of the lumbar spine showed no fracture and the diagnoses were lumbar strain and cervical strain. It was stated he was given medications and was to be checked by a doctor again on December 6, 2004. Handwritten on the form are some work restrictions that appear to be a 10 pound lifting restriction and he was to avoid repetitive bending, stooping or prolonged standing or sitting and he was to do no climbing. (It is somewhat hard to read this form.)

There is a typewritten work status report prepared by Dr. B on December 3, 2004 noting the diagnoses were a lumbar and cervical strain, and this was the first visit for the injury and it was stated he has had x-rays and was given medication. It was felt he could do modified work with a number of restrictions.

From that same facility there is a work status report prepared by Dr. W. dated December 7, 2004. The same diagnoses are listed and it was stated he was improving slowly and he could be at modified work with a number of restrictions.

Another report from Dr. W of December 13, 2004 again states he could be at modified work with the same restrictions. It was stated he was improving but slowly.

Next from that facility there is a report of an orthopaedic consultation prepared by Dr. Y. who examined the patient on December 30, 2004. In this report it was stated he had worked for the last 10 years for the company as a machine operator. (Please note: He told me he was hired in June or July 2004, a few months prior to the industrial injury.)

The history of injury of catching a falling cabinet is noted, with the development of an immediate pull and sharp pain in the low back. It was stated he took some Tylenol and felt better but the pain started increasing and he reported it on December 2 and was seen at the care station on December 3, 2004. He was given medication. It was stated he was having ongoing pain and was referred for further evaluation. He denied prior injuries to the neck or back. He was now complaining of severe pain, mostly in the low back, aggravated by prolonged sitting or standing. He could not arise from a sitting position on his own because of pain. Tylenol and other medications had helped only temporarily. The pain was not radiating beyond the buttocks. He had difficulty sleeping. There was some sharp pain and heaviness in the neck area that did not radiate into the arms but the neck pain was minor compared to the low back. He was currently not working.

Physical examination revealed he was in moderate distress with pain, and his gait was very slow and antalgic on the left, but normal reciprocating. He could toe and heel walk. Neck range of motion was completely normal. There was some tenderness of the upper trapezius, but no spasm. Lower back examination revealed decreased lordosis and tenderness and moderate to severe spasm. There was limited motion in all directions (but the motion was much greater than that demonstrated by the patient to the undersigned currently). It was noted leg raising was negative at 90 degrees bilaterally with negative stretch signs. Neurological examination as described revealed symmetrical reflexes and no loss of strength or sensation. It was noted x-rays of the lumbar spine were normal and the diagnosis was a severe lumbosacral strain and a mild cervical strain. It was noted that his findings were consistent with a strain and the examination showed no evidence of a radicular component. It was felt he should have conservative

treatment with medication and a course of physical therapy. He was released to modified work with a lot of restrictions.

A brief work status report from Dr. Y of December 30, 2004 notes that he was given Motrin and Parafon Forte and was to have a home interferential unit and be at modified work. He was also to start physical therapy three times weekly.

A physical therapy evaluation dated January 6, 2005 notes the patient was complaining of constant severe pain in the mid lumbosacral spine with tingling and numbness to the left lower extremity and throbbing pain in the neck without radiation. The pain appeared to have been aggravated since injury. He was to have a treatment program with a pool program, modalities and other treatments.

On January 20, 2005 a work status report from Dr. Y notes he was still being followed with the same diagnoses, something is mentioned about getting an MRI study but I cannot read the handwritten note clearly, and he was still to be at modified work.

There is a report of an MRI study of the cervical spine that was performed on February 3, 2005. There was a 2-3 mm disc protrusion at C6-7 with no neurological involvement. (Please note: An isolated disc protrusion at one level in the neck would not be related to a straining injury that involved the lower back.)

A follow-up report from Dr. Y is dated February 10, 2005 and notes the patient was complaining of increasing pain and there was now pain radiating down the legs and he wanted a back support. There was numbness and tingling. He was taking over-the-counter Tylenol because the other medicines had upset his stomach. It was stated therapy was only helping “somewhat.” He was not working. Neck range of motion was normal, although it was noted that with extension there was “exaggerated facies.” Lumbosacral motion revealed only a 10 degree limitation of flexion and extension, and extension was painful. Other motions are normal. The neurological examination as described was normal. MRI findings were noted. It was stated there was also an MRI of the lumbar spine (not available to the undersigned) that showed mild desiccation at L3-4 with a mild bulge that did not impinge on neural elements. The diagnoses of Dr. Y were “Cervical and lumbosacral strain. Subjective complaints much greater than any objective findings.”

Under the discussion it was noted that although he still had significant subjective complaints, they were not backed by objective findings. It was noted the MRI showed the spine was “structurally sound with no evidence of any danger to any of the neural elements or the spine itself.” It was explained to the patient that the best thing to do would be to exercise and he was released to regular duty.

The only other report available is a very brief note from I. Boyarsky, D.O. which is a disability status report and not a full medical report. It was stated the patient was seen on March 22, 2005 and he was temporarily disabled from that date through April 28, 2005, and that he needed treatment. No diagnosis is made and no examination findings are reported.

This concludes the review of the submitted records.

Comment

After a review of the history as obtained directly from the patient and the records that have been submitted, my physical examination and the x-rays available, it is my opinion that the above-captioned patient has the following orthopaedic conditions:

1. History of cervical strain.
2. History of lumbar strain.

It has been more than six months since this patient's injury, and instead of showing any gradual improvement, his symptom complexity is expanding. I would concur with the final impression of Dr. Y that the best management program for this patient would be a progressive exercise program. I do not believe that intensive physical therapy treatments that are not reducing his pain are indicated, necessary or likely to be of any benefit.

I believe that for all practical purposes, it would be in the best interests of the patient and all parties concerned to consider his condition is at MMI at this time and settle his industrial injury claim.

The patient's condition has reached MMI subsequent to January 1, 2005, and will be rated according to the AMA Guides to Evaluation of Permanent Impairment, 5th Edition.

The patient's neck injury falls into the DRE cervical Category I.

It is noted there are no significant clinical findings. There is no muscle guarding or spasm. He has full range of motion, although he complains of pain at the extremes. He has some non-organic findings including the marked low back pain even prior to the application of any pressure with cervical compression. His sensory changes do not follow any organic neurological distribution. The motor function is intact, reflexes are normal and grip strengths are appropriate.

I see no evidence of permanent impairment and no objective signs. I recommend this patient do appropriate stretching and neck exercises before and after vigorous activity.

This patient's neck condition should be rated as 0% impairment of the whole person as per the AMA guides (pages 392).

With reference to his lower back condition, I would rate his impairment in the DRE category of DRE lumbar Category II.

It is noted his history and examination are consistent with a specific injury on a specific date. At the time of the examination he has marked restriction of motion; however, this appears to be more voluntary than objective impairment. He will not even attempt to perform any flexion. This is in contrast with his initial examination which shows excellent motion. His leg raising signs do not confirm any radicular finding and he has non-verifiable radicular complaints. There is no alteration of the structural integrity and no significant radiculopathy.

DRE Lumbar Category II rates to 5% to 8% impairment of the whole person, and I believe that the non-objective factors which predominate would put this patient in a 5% category of whole person impairment.

Using the combined value chart of a 5% impairment plus a 0% impairment results in a 5% whole person impairment for this individual.

At the current time, I do not believe that any ongoing medical care is indicated or necessary but the patient should be encouraged to pursue a regular home exercise program. He should be reassured regarding the absence of any significant pathology in his back or neck.

Giving the patient every benefit of the doubt, I believe that at most he should have a permanent work restriction in the category that would preclude very heavy lifting. Whether or not he could return to his usual and customary work would best be determined by reviewing an analysis of the physical demands of his usual and customary work. If an analysis can be obtained, I would be happy to review it and provide a supplemental report.

In terms of apportionment based upon causation, the information available indicated this patient has only had one injury in his lifetime. There is no evidence of any other factors contributing to his disability. Therefore, I would apportion all of his current condition to industrial causation, namely the lifting and straining injury that occurred at work on November 30, 2004.

The patient was seen at this time for orthopaedic evaluation purposes only. If there are any questions regarding these opinions, please do not hesitate to call upon me.

Please note, obtaining a history directly from the patient (with the assistance of an interpreter, if needed), reviewing and summarizing all records (if any), performing the entire physical examination and all measurements, reviewing x-rays (if any), and the preparation and dictation of this entire report were entirely and solely performed personally by the undersigned. The only exceptions were the measuring of vital signs and the transcription of my personally dictated findings onto an office work sheet which were done by (office manager), who has been my office manager and back office assistant for 20 years and has been instructed and monitored in these activities by me. The time spent in connection with this evaluation was in compliance with any available guidelines.

I declare under penalty of perjury that there has not been a violation of Labor Code Section 139.3 that the contents of the report are true and correct to the best of my knowledge, and any statements concerning any bill for services are true and correct to the best of my knowledge.

I declare under penalty of perjury that the information contained in this report and its attachments, if any, is true and correct to the best of my knowledge and belief, except as to the information that I have indicated I have received from others. As to that information, I declare under penalty of perjury that the information accurately describes the information provided to me and, except as noted herein, that I believe it to be true.

Date_____ in the County of_____

Sincerely yours,

Joe AME
Diplomat of American Board of Orthopaedic Surgery

CC: Adjustor
Defense Attorney
Applicant Attorney

Fortunes Rocks Consultants MD

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Independent Medical Evaluation Report

Examinee: Tom Sample
File/Claim Number: 000123456

Date of Examination: September 18, 2004
Examining Physician: William F. Boucher, MD
Examination Location: Vermont

Date of Birth: January 1, 1964
Date of Injury: December 18, 1985

Client Organization: The Best Insurance Company
Referral Source: Miss Adjuster

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Introduction

This 40-year old, right-handed man was referred for an independent medical evaluation (IME) by the above client. The independent medical examination process was explained to the examinee, and he understands that no patient/treating physician relationship was established. Mr. Sample was advised that the information provided will not be confidential and a report will be sent to the requesting client. The client's questions were described to the examinee.

Mr. Sample was cooperative. History was provided by the examinee who was a good historian. The information he provided was consistent with the medical records provided. Accompanying Mr. Sample was his wife who waited outside.

A questionnaire and pain inventories were completed by the examinee. To ensure accuracy, the clinical history was dictated in his presence.

Mr. Sample reported no difficulties occurring during the examination.

The client provided the following clinical records: The Medical Center and One Memorial Hospital.

These records were carefully reviewed, and listed for the examinee. No records prior to December 18, 1985 or subsequent to June 14, 2004 were available to review. These records will be purged from this file. The report, questionnaires, pain inventories and other material specific to this evaluation will be retained. The following records were not available at the time of this evaluation: None.

History

Preexisting Status

Mr. Sample denies any previous problems or injuries, including any other work- or liability-related injuries. Mr. Sample also denies having any difficulties similar to those he is now experiencing prior to the date of injury.

Injury

Mr. Sample reports that on December 18, 1985, he fell on the ice landing on his left hip.

At that time the difficulties were fracture of the left hip.

Clinical History

The examinee was transported to the Hospital Emergency Department, then transported at his request to One Medical Center. Examination at that time revealed shortening and external rotation of the left lower extremity, pain with internal and external hip rotation. X-ray revealed a left intra trochanteric fracture. The examinee was seen by Dr. Kinko, who on December 19, 1985, performed open reduction and internal fixation of the fracture. The examinee did have postoperative complications including pulmonary edema, which resolved promptly with treatment.

The examinee was then discharged with gradually increasing weight bearing. By February 2, 2004, the examinee was ambulating with a cane. He was referred to physical therapy. Initial evaluation revealed mildly decreased left hip motion.

By mid March 2004, Dr. Kinko noted essentially normal gait with painless hip motion. Hip motion was not measured at that time. The examinee's left leg was noted to be one-half inch short and the patient was given a lift.

By mid June 2004, x-rays revealed healing of the fracture with some ongoing impaction. Gait was normal. The examinee was released from Dr. Kinko's care at that time. The examinee has required no further treatment for his left hip condition.

Diagnostic Studies:

Study	Date	Result
X-Ray Left Hip	6/18/03	Fixation hardware in good position; fracture healed with slight impaction

Current Status

The examinee's chief concern is whether it is a "permanent injury."

He reports difficulties with pain which is primarily located in the left hip. The pain is described as minimal. The pain is worsened by lying on the left side and prolonged walking. The pain is reported as intermittent.

On a scale from 0 (no pain) to 10 (excruciating pain), the examinee reports the pain now is a 2. During the past month the pain averaged 2, with a high of 4 and a low of 1.

Functional Status

Examinee states he is able to lift a gallon of milk; able to lift a heavy bag of groceries; able to lift a pail of water. Examinee states that he can sit for up to two hours, stand for up to two hours and walk for up to two miles.

Occupational History

At the time of the injury, he had been employed by The Fort at #4 and had been working there for two years. He was working full-time as a museum administrator. According to the description provided by the examinee, the job involved operation of a museum and staff. He has a college education.

In terms of current work status, he is currently not working. He last worked on January 12, 2004.

Social History

The examinee lives in Charleston, NH. The activities of a typical day include "seeking new job." He denies performing any other work activities or vigorous recreational pursuits.

The examinee does not smoke or drink.

Past Medical History

Medical:	Chron's disease
Surgery:	Bypass surgery
Medications:	See list
Allergies:	None known

Review Of Systems

Noncontributory

Family History

Noncontributory

PHYSICAL EXAMINATION

Observations

The examinee is a well-developed, well-nourished male. He appears healthy. Examination of the hands reveals mild callus. No assistive devices were used. Height was 5 feet, 7 inches and weight 160 pounds as reported by the examinee.

Behavioral Observations

The examinee was pleasant, cooperative and attentive. Affect was normal. During the visit, he appeared comfortable. He sat continuously for up to 20 minutes. There was no significant pain behavior. Non-physiologic findings were not present.

Structural Examination

In the standing neutral position, cervical, thoracic and lumbar curves were well-maintained. The shoulders were symmetric. The right PSIS was elevated (as the examinee was not wearing his heel lift). Gait was normal with no antalgia. There was no external rotation of the left foot.

Lower Extremity Examination

Inspection revealed a well-healed surgical scar over the lateral left hip. Hip motion as measured with a goniometer was symmetrically decreased in external rotation, but otherwise normal.

MOTION	NORMAL	LEFT	RIGHT
Forward flexion	100	120	120
Extension	0	0	0
Abduction	25	35	35
Adduction	15	30	30
Internal Rotation	20	35	35
External Rotation	30	15	15

The examinee had no pain with hip motion. Resisted hip abduction and external rotation was pain free. There was no tenderness of hip structures. There was no left thigh atrophy. However, leg length measured from the lateral pelvic rim to the lateral tibial plateau revealed a 2.5 cm deficit on the left.

PAIN STATUS INVENTORIES

Pain Drawing

The examinee completed a pain drawing (attached), using symbols to describe sensations. This drawing did reveal findings suggestive of symptom magnification.

Pain Disability Index

The Pain Disability Index uses rating scales to measure the extent of perceived disability in seven areas of life. The results are as follows:

Area	Perceived Disability
Family/home responsibilities	20%
Recreation	20%
Social Activity	0%
Occupation	10%
Sexual activity	40%
Self-care	0%
Life-support activities	0%

The total score is 9 out of a possible 70, for a total index of 13%. This indicates a mild degree of perceived disability.

CES-D

The Center for Epidemiologic Studies Depressed Mood Scale was administered. The examinee scored 10, which is not consistent with a depressed mood.

CONCLUSIONS

Diagnoses

1. Left hip fracture, intra trochanteric
   1.1 Status post open reduction and internal fixation
2. Status post fracture left ankle
3. Coronary Artery Disease
4. Diabetes
6. History depression

Discussion

The examinee's injury of December 18, 1985 was a left hip fracture which did not involve the hip joint. He underwent open reduction and internal fixation (without joint replacement) and has done very well. He does generally use a heel lift due to left leg shortening, but is currently walking two miles per day with no difficulty.

Causation

Based upon the available information, to a reasonable degree of medical certainty, there is a causal relationship between the examinee's current left hip complaints and the reported injury. The examinee's more diffuse low back symptoms are of uncertain origin, given the degree of symptom magnification present. More likely than not, these symptoms are due to pre-existent (mild) facet arthropathy, obesity and symptom magnification.

Work Capacity

This examinee has at least a sedentary work capacity as defined in the *Dictionary of Occupational Titles*, U.S. Department of Labor. At this time, the examinee can lift perhaps 10 pounds occasionally and five pounds frequently. He assumes that he is unable to sit, but I can find no objective reason why that should be the case. In truth, it is likely that he could sit and stand for reasonable periods of time, requiring frequent position changes. He could walk for perhaps 15 minutes per hour. He can bend and twist occasionally, but should involve spotting. He can kneel and crawl occasionally and climb stairs minimally. He should avoid ladders. He can push and pull with perhaps 20 pounds effort. He would have no other restrictions regarding upper extremity use. He should avoid driving a motor vehicle or operating machinery due to her multiple sedating medications.

Prior to his July 2004 hip surgery, he would have had essentially the same work restrictions, except that his duration for sitting, standing and walking may have been somewhat greater.

Appropriateness of Care

The client has asked that I specifically address the issue of appropriateness of medical care. Based on the specifics of this care, it is my professional opinion that care has not been entirely consistent with the usual standards of care for this problem. In particular, the use of Avinza is inappropriate given the lack of objective reason for the examinee's pain complaints and the ineffectiveness of the medication. Furthermore, use of Neurontin has been inappropriate given the lack of evidence of neuropathic pain. Finally, the failure to address the examinee's probable significant depression has been inappropriate.

Prognosis

The overall prognosis is good.

Maximum Medical Improvement

The examinee has achieved maximum medical improvement. MMI is defined as the date after which further recovery and restoration of function can no longer be anticipated, based upon a reasonable degree of medical probability.

Permanent Impairment Evaluation

Permanent impairment evaluation was performed in accordance with the AMA *Guides to the Evaluation of Permanent Impairment*, Fifth Edition. The examinee warrants impairment on the basis of a 2.5 cm limb length discrepancy. According to Table 17-4 (page 528), this imparts 7% lower extremity impairment, which converts to 3% whole person impairment.

Recommendations

Diagnostic/Consultation

No further diagnostic testing or consultation is indicated.

Therapeutic

The examinee will continue to require a heel lift of perhaps 1.5 cm. No other treatment will be necessary for his left hip condition.

Qualifications

I am a founding member of the American Board of Independent Medical Examiners, having served on the Selection Committee, and am certified by ABIME. I have performed over 2000 Independent Medical Examinations and Permanent Impairment Ratings in the past 13 years. I have spoken to national groups on work fitness and disability issues, and have built a national reputation for my expertise in these issues. I have been a contributor to publications regarding disability issues and permanent impairment. I am board certified in Occupational Medicine, and am a Fellow of the American College of Occupational and Environmental Medicine and the American College of Preventative Medicine. I am past Chairman and current secretary of ACOEM's Work Fitness and Disability Evaluation Section. I maintain a clinical practice as Medical Director of WorkWell at Southern Maine Medical Center.

The above analysis is based upon the available information at this time, including the history given by the examinee, the medical records and tests provided, the results of pain status inventories, and the physical findings. It is assumed that the information provided to me is correct. If more information becomes available at a later date, an additional report may be requested. Such information may or may not change the opinions rendered in this evaluation.

The examiner's opinions are based upon reasonable medical certainty and are impartial. Medicine is

both an art and a science, and although an individual may appear to be fit for work activity, there is no guarantee that the person will not be re-injured or suffer additional injury. If applicable, employers should follow the processes established in the Americans with Disabilities Act, Title I. The opinions on work capacity are to facilitate job placement, and do not necessarily reflect an in depth direct threat analysis. Comments on appropriateness of care are professional opinions based upon the specifics of the case, and should not be generalized, nor necessarily be considered supportive or critical of, the involved providers or disciplines.

Any medical recommendations offered are provided as guidance, and not as medical orders. The opinions expressed do not constitute a recommendation that specific claims, or administrative action be made or enforced.

Sincerely,

William Boucher, M.D., C.I.M.E.

WB/ls

ACRODOCZ
Organized Medical Docz

Chronological
Table Summary

Client Name: Mr. John Doe
No. of Pages: 100
DOI: December 1, 2022
Treatment Dates: December 2, 2022 to February 12, 2023

CONFIDENTIAL ATTORNEY WORK PRODUCT

Table Summary: Mr. John Doe

DOS	Facility/Provider Name	Description	Page Ref.
12/02/2022	Christian Hospital Northwest/Michael Brickman, RN/Kimberly Janean King, NP/Robert Emmons Abbott, MD	**Emergency Department Consultation Note** **Chief complaint:** Motor vehicle crash. Mr. [redacted] was a 40-year-old male who was involved in a motor vehicle accident a night prior to this visit. He was a restrained passenger of a vehicle that was struck on the front driver's side. There was full airbag deployment. He complained of pain to the left side of his body, right knee pain, and pain across his upper and lower back after the accident. There was no loss of consciousness, nausea, vomiting, numbness, tingling, weakness, wounds, chest pain, shortness of breath, or abdominal pain. He did not take anything for his symptoms. Review of systems was positive for arthralgias on the right knee and left side of the body as well as upper and lower back pain **Past medical history:** Covid-19, sinusitis, ulcerative colitis, and vertigo. **Past surgical history:** Fracture surgery (2016) and brain surgery. **Problem list (as of 12/02/2022):** Alcohol use (07/14/2022); appendicitis (07/14/2022); constipation (07/14/2022); contusion of chest (06/26/2022); contusion of right calf (06/26/2022); contusion of right elbow (06/26/2022); gastroesophageal reflux disease without esophagitis (07/14/2022); generalized abdominal pain (07/14/2022); motor vehicle collision, initial encounter (06/26/2022); nausea and vomiting (07/14/2022); pansinusitis (01/22/2021); primary hypertension (07/14/2022); skin burn (07/14/2022); sprain of right shoulder, right (06/26/2022); strain of lumbar region (06/26/2022); and tobacco use (07/14/2022).	4-37

		Physical exam: • Head was normocephalic with no signs of trauma. • Generally normal range of motion of the musculoskeletal system. • Normal range of motion of the cervical back. Neck was supple. There was tenderness to the bilateral trapezius with palpation. No bony tenderness was noted. There was also tenderness to the left paraspinal areas with palpation. No spinous process tenderness was appreciated. • The thoracic back was nontender and with normal range of motion. • The bilateral lumbar back was tender on palpation. No bony tenderness was noted. Range of motion was full. • The right knee had no swelling, effusion, erythema, ecchymosis, or lacerations. Range of motion was normal. There was generalized tenderness to palpation. Alignment and pulse were normal. • Neurologic exam was unremarkable. **Procedures:** X-rays of the right knee and lumbar spine were performed. **Medications administered:** Ibuprofen and acetaminophen. **Diagnoses:** • Motor vehicle accident, initial encounter • Strain of trapezius muscle, unspecified laterality, initial encounter • Contusion of right knee, initial encounter **Plan:** • Prescriptions for the following pain medications were provided:	

		acetaminophen, methocarbamol, and naproxen. • He was discharged home in a stable condition.	
12/02/22	Christian Hospital Northwest/Paul Hargan, MD	**X-Ray of the Right Knee** **History:** Right knee pain after MVA. **Impression/Findings:** • There was no sign of any acute osseous or articular abnormality. • There were no specific soft tissue abnormalities.	38-39
12/02/2022	Christian Hospital Northwest/Paul Hargan, MD	**X-Ray of the Lumbar Spine** **History:** Status post MVC, back pain. **Findings:** • The lumbar vertebral bodies and the disc spaces were normal in width. • In the lumbar vertebra, there was no evidence of compression deformities or spondylolisthesis. **Impression:** • Unremarkable lumbar spine study.	40-41
12/13/2022	Ortho Sport and Spine Physicians/Nicole Lane, NP-C/Jeffrey Lee, DO	**Orthopedics Consultation Note** **Chief complaints:** Neck pain, low back pain, and right knee pain. Mr. [redacted]. complained severe neck, low back, and right knee pain. He was involved in a motor vehicle accident on 12/01/2022. He denied having any pain prior to the accident. He described his neck pain as an aching tightness from the posterior and was radiating down his bilateral shoulders and bilateral scapular region,	86-89

		left side worse than right. He was having muscle spasms daily. He was also having a new onset of headaches. He denied numbness, tingling, or weakness in his upper extremities. His low back pain was described as an aching pain aggravated by increase in physical activity. Activities such as lifting, standing, bending, and sitting for prolonged periods of time were difficult for him. He was also having difficulty sleeping at night. His right knee pain was located along the patellar tendon and medial and lateral joint lines. He was having pain with going up the stairs, squatting, or lifting. His pain was described as sharp and shooting that could make his knee feel buckled at times. He has not had any MRI imaging, physical therapy/chiropractic therapy, or injections at this point. He rated his right knee pain as a 5/10 and his neck and low back pain as a 7/10. **Review of systems:** Positive for weight loss, change in appetite, change in vision, stomach pain/ulcers, constipation, blood in stools, neck pain, back pain, muscle aches, painful joints, and headaches. **Past surgical history:** Boxer fracture – plates and pins in the hand (2017). **Physical exam:** • Cervical spine: Cervical lordosis was normal and maintained. There was limited range of motion on extension and flexion. There was pain with flexion and extension as well as lateral rotation bilaterally. There was tenderness along the paraspinal muscles bilaterally, left worse than right side. There was a small palpable trigger point noted	

		along his left scapular region. Motor strength exam was 5/5 in all muscle groups. Deep tendon reflexes were 2/4 bilaterally at brachioradialis and biceps. Spurling sign was positive bilaterally. Clonus was negative bilaterally. Phalens was negative bilaterally. • Lumbar spine: Lumbar lordosis was normal and maintained. There was limited range of motion with flexion and extension. There was pain in flexion. No obvious pelvic obliquity was noted. There was no tenderness on the lumbar spinal processes. There was tenderness on palpation to bilateral paraspinal muscles. Positive facet loading sign with flexion was noted. Motor strength was 5/5 bilaterally in all muscle groups. Deep tendon reflexes were 2/4 with bilateral patellar and achilles. There was tenderness on palpation over the bilateral sacroiliac joints. Straight leg raise was negative bilaterally. Faber exam was negative bilaterally. Thigh thrust and compression exam was negative bilaterally. • Right knee: Gait pattern was normal. There was a 1+ effusion to the anterior aspect of the right knee. There was no erythema or ecchymosis. Bench testing of the right knee revealed 5 degrees of extension and 115 degrees of flexion. There was tenderness on palpation to the medial and lateral joint lines. He was stable to valgus stress. There was slight laxity with varus stress to the right knee. ACL instability - Lachman exam was negative. PCL instability - Posterior drawer test was negative. Meniscus tear - McMurray exam was not tested due to his complaint of severe pain. Pulses were 2+	

		bilaterally. Motor strength and sensation were intact bilaterally. **Assessment:** • Cervicalgia • Myofascial muscle pain • Trigger point of neck • Lumbago • Right knee pain **Imaging studies taken in this visit:** • X-ray of the cervical spine showed straightening of the cervical lordosis, likely paraspinal muscle spasm. Disc heights were well-maintained without evidence of osteophyte formation, spondylolysis. There was no acute fracture present. • X-ray of the lumbar spine showed straightening of lordotic alignment, likely paraspinal muscle spasm. There was severe disc narrowing at T10-11, T11-T12, T12-L1, L5-S1 most severe at L5-S1. There was no evidence of any acute fracture. • X-ray of the right knee showed overall good alignment. There was no evidence of acute fracture. There was some mild narrowing in the medial joint line and the patellar femoral joint. There was no osteophyte formation. The patella was slightly lateralized in alignment on the sunrise view. **Procedure performed in this visit:** • A trigger point injection to the left scapular region was performed using a combination of bupivacaine and Kenalog which Mr. [redacted] was able to tolerate well.	

		Plan: • MRIs of the cervical spine, lumbar spine, and right knee were ordered. • Dendraxin Neurodendraxcin Lotion to be applied to affected areas, cyclobenzaprine, LSO brace, and TENS Therapy Pain Relief Device, and knee brace with hinged bars were recommended and dispensed at this clinic.	
02/07/2023	Ortho Sport and Spine Physicians/Nicole Lane, NP-C/Jeffrey Lee, DO	**Orthopedics Follow-Up Consultation Note** Mr. [redacted] reported that he did not have any relief since his last visit. His main complaints were continued low back and neck pain. His pain was aggravated by physical activity. Bending, twisting, and lifting motions were difficult. He was having difficulty making it through a workday without pain. He rated his pain as a 7-8/10. He has not scheduled his MRI yet. He also has not had any physical therapy/chiropractic therapy. **Physical exam:** • Cervical spine: Cervical lordosis was normal maintained. There was limited range of motion on extension and flexion. There was pain with flexion and extension as well as lateral rotation bilaterally. There was tenderness along the paraspinal muscles bilaterally, left worse than right side. There was a small palpable trigger point noted along his bilateral scapular region, left worse than right side. Motor strength exam was 5/5 in all muscle groups. Deep tendon reflexes were 2/4 bilaterally at brachioradialis and biceps. Spurling sign was positive bilaterally. Clonus was negative bilaterally. Phalens was negative bilaterally.	90-92

		• Lumbar spine: Lumbar lordosis was normal and maintained. There was limited range of motion with flexion and extension. There was pain in flexion. No obvious pelvic obliquity was noted. Lumbar spinal processes were nontender. There was tenderness on palpation to bilateral paraspinal muscles. Positive facet loading sign with flexion was noted. Motor strength was 5/5 bilaterally in all muscle groups. Deep tendon reflexes was 2/4 with bilateral patellar and achilles. There was tenderness on palpation over the bilateral sacroiliac joints. Straight leg raise was negative bilaterally. Faber exam was negative bilaterally. Thigh thrust and compression exam were negative bilaterally. **Assessment:** • Lumbago • Cervicalgia • Myofascial muscle pain **Procedure performed in this visit:** • Trigger point injections to the two palpable points along the scapular region were performed using a combination of xylocaine and Kenalog. He was able to tolerate this procedure well. **Plan:** • Dendracin Neurodendraxcin Lotion and TENS Therapy Pain Relief Device were refilled. • Physical therapy referral was arranged. Recommendation was dry needling as needed for trigger points. Frequency and duration of visits were recommended to be at 1-2 times a week for 6 weeks.	

		• MRIs of the cervical spine and lumbar spine were ordered. • It was recommended that he continue with conservative care including his LSO brace, medications, and TENS unit.	
02/12/2023	St. Lukes Rayus Chesterfield/Alex Georges, MD	**MRI of the Right Knee (without contrast)** **History:** Knee pain. **Findings:** • Within the medial compartment, the meniscus and articular cartilage were normal. There was minimal increased T2 signal about the medial aspect of the medial femoral condyle without a fracture line. • Within the lateral compartment, there was no meniscal tear. The articular cartilage was normal. • Within the patellofemoral joint, the articular cartilage was normal. • The anterior and posterior cruciate ligaments were intact. The medial collateral and lateral collateral ligament complex were normal. The extensor mechanism was normal. • There was a small joint effusion. There was trace Baker's cyst. The marrow signal was normal. **Impression:** • Minimal nonspecific marrow edema of the medial aspect of the medial femoral condyle that might represent a small contusion. There was no fracture. • Small joint effusion of the right knee.	95-96

02/12/2023	St. Lukes Rayus Chesterfield/Yihua Zhou, MD	**MRI of the Lumbar Spine (without contrast)** **History:** Low back pain and bilateral knee pain. **Findings:** • There was a transitional lumbosacral segment with a presumed partially sacralized L5. If absolute confirmation of the patient's spinal numbering was necessary from a clinical standpoint, radiographic views of the thoracic and lumbar spine were suggested. • The alignment was normal. Vertebral bodies were normal in height without evidence of compression fractures. Marrow signal intensity was normal. The conus medullaris was terminating at the level of T12/L1 and the distal spinal cord signal intensity was normal. The intervertebral discs were normal in height. • No soft tissue abnormality was identified. • L1-L2: There was no disc bulge. There was no central canal stenosis. There was no facet osteoarthritis. There was no neural foraminal stenosis. • L2-L3: There was no disc bulge. There was no central canal stenosis. There was no facet osteoarthritis. There was no neural foraminal stenosis. • L3-4: There was no disc bulge. There was no central canal stenosis. There was no facet osteoarthritis. There was no neural foraminal stenosis. • L4-L5: There was mild diffuse disc bulge and a small central disc protrusion. There was no central canal stenosis. There was mild bilateral facet osteoarthritis. There was no neural foraminal stenosis.	97-98

		• L5-S1: There was no disc bulge. There was no central canal stenosis. There was mild bilateral facet osteoarthritis. There was no neural foraminal stenosis. **Impression:** • Mild multilevel degenerative disc and joint disease at L4-L5 and L5-S1 as detailed above. No central canal or neuroforaminal stenosis.	
02/12/2023	St. Lukes Rayus Chesterfield/Kishan Yalavarthi, MD	**MRI of the Cervical Spine (without contrast)** **History:** Neck pain, motor vehicle injury. **Findings:** • There was straightening of the normal cervical spine lordosis, without acute fracture, or marrow edema. There was normal signal in the cervical spinal cord and craniocervical junction. The cerebellar tonsils were normally positioned, without Chiari malformation. • C2-3: Within normal limits. • C3-4: There was disc bulging, without focal herniation, canal, or foraminal stenosis. There was mild bilateral facet disease. • C5-6: There was no disc bulging, herniation, canal, or foraminal stenosis. There was mild left facet disease. • C6-7: There was disc bulging, with a superimposed right central and subarticular 5 mm AP disc herniation with an associated annular tear with 6 mm of caudal migration. There was right ventral cord impingement, with moderate to severe central canal stenosis with the AP diameter of the canal at 7 mm. there was bilateral uncovertebral joint and facet disease, with	99-100

		mild left and moderate right foraminal stenosis. • C7-T1: There was no disc bulging, herniation, canal, or foraminal stenosis. **Impression:** • C6-7 right central and subarticular disc herniation with caudal migration with right ventral cord impingement, and moderate to severe central canal stenosis at this level.	

Question about your bill? Please contact Customer Service at 314-653-5900 or 618-463-7383

Guarantor: [redacted]
Patient: [redacted]
Date of Birth: [redacted]
Account ID: [redacted]
Statement Date: 03/15/23

Location: Christian Hospital
Tax ID: [redacted]
Physician: Abbott, Robert
Service Dates: 12/02/22 - 12/02/22

This is not a bill. This is an itemized statement.

Hospital Charges

Svc Date	Rev Code	CPT/ HCPCS	Procedure	BJC HB DB Charge Description	Qty	Charge
12/02/22	0320	72110	182605	HC Spine Lumbar Min 4 Views	1	608.00
12/02/22	0320	73564	182685	HC Knee Comp Min 4 Views	1	391.00
12/02/22	0450	99284	252550	HC ED Level 4 Intermedate+Proc	1	3,082.00
Total hospital charges:						**4,081.00**

Hospital Payments and Adjustments

Date	Description	Amount
12/30/22	United HealthCare Payments Copay: 500.00	-989.00
12/07/22	United HealthCare Adjustments	-2,592.00
01/17/23	Home State Health Plan Payments	-202.86
02/09/23	Home State Health Plan Adjustments	-297.14
Total hospital payments and adjustments:		**-4,081.00**

Rev Code	Description	Amount
0320	Radiology - Diagnostic - General Classification	999.00
0450	Emergency Room - General Classification	3,082.00

BJC HealthCare

Hospital Account Number:	202771461
Hospital Account Amount Due:	0.00

[redacted]

BJC HealthCare
Christian Hospital NorthWest
HealthCare
1225 Graham Road
Florissant MO 63031-8012

[redacted]
MRN: [redacted] DOB: [redacted] Sex: M
Acct #: [redacted]
Visit date: 12/2/2022

Care and staffing during this admission was provided during a governmental declaration of emergency, and during a time in which surge operation plans were invoked to respond to the volume of admissions related to the COVID-19 pandemic. This process began on 8/6/21 and continues. Surge Documentation Standards implemented on 1/11/22 at 6 a.m. and ended 3/9/22 at 7 a.m.

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

BJC HealthCare

ADMISSION FORM

Encounter #	MR#	Enc Start Date Time	Pt Location	
202771461	[redacted]	12/2/2022-N/A		

CSN#	LOC	Clin Svc	PAT	Enc Type
[redacted]				Emergency

Adm Dx Code Desc	Inf Dis	Visitor Restriction

Confidential	Adm Type	Adm Source	Last Enc Date
No	Emergency	1	

Birth Date - Age	Race	Ethnicity	Maiden Name
[redacted]	Black or African American	Non-Hispanic	

Gender	Marital	Religion	Soc Sec #
Male	Single	CHRISTIAN	[redacted]

Preferred Language	Birth Place	Health Care Proxy Sts	Patient Living Will Status
English			

Patient Name Address, Phone	Employer Name, Address, Phone	Emplm Sts
[redacted] Home: [redacted] Work: Mobile: [redacted]	,	NOT EMPLOYED

Guarantor Name, Pt Rel, Address, Phone	Guarantor Empr, Address, Phone	Emplm Sts
[redacted] Self Phone: [redacted] Work:	,	NOT EMPLOYED

EMERGENCY CONTACTS

Name	Home Phone	Work Phone	Mobile Phone	Relationship	Lgl Grd
[redacted]	[redacted]		[redacted]	Mother	No

Insurance 1 Name, Address, Phone

UNITED HEALTHCARE/UHC CHOICE PLUS

,

877-842-3210

Verified?

Policy Number	Group Number
[redacted]	[redacted]

Referral/Authorization #

Eff. Date	Pre-Cert. Phone
12/1/2021	

Subscriber DOB

[redacted]

Subscriber Employer

Insurance 2 Name, Address, Phone

CENTENE/HOME STATE HEALTH P*

PO BOX 4050
FARMINGTON, MO 63640-3829

855-694-4663

Verified?

Policy Number	Group Number
[redacted]	

Referral/Authorization #

Eff. Date	Pre-Cert. Phone
12/2/2022	

Subscriber DOB

[redacted]

Subscriber Employer

Insurance 3 Name, Address, Phone

/

,

Verified?

Policy Number	Group Number

Referral/Authorization #

Eff. Date	Pre-Cert. Phone

Subscriber DOB

Subscriber Employer

Insurance 4 Name, Address, Phone

/

,

Verified?

Policy Number	Group Number

Referral/Authorization #

Eff. Date	Pre-Cert. Phone

Subscriber DOB

Subscriber Employer

Primary Physician	Referring Physician	Admitting Physician, ID	Attending Physician, ID
No, Physician 999-999-9999	King, Kimberly Janean, NP	No admitting provider for patient encounter.	No att. providers found

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Incident Date	Incident Type	Inc St/Prov	Incident Description / Location

FOR EMS ONLY:

Visit Information

Appointment Information

XR KNEE RIGHT 4+ VW **Completed**
12/2/2022 4:00 PM

	Time	Provider	Department	Length
	4:00 PM	NW XR ED	CH NW DIAG IMG	15 min

Referral Provider: KING, KIMBERLY JANEAN
Enc Form Number: 24823587
Arrival Time: 3:57 PM

History

Made On:	12/2/2022 3:57 PM	By:	Dunham, Carrie An M, RT	RIS
Checked In:	12/2/2022 3:57 PM	By:	Dunham, Carrie An M, RT	ES
Lnk Req:	12/2/2022 3:57 PM	By:	Dunham, Carrie An M, RT	Vrfy Orders
Remove Arr.:	12/2/2022 5:12 PM	By:	Hopper, Teresa L., RN	MR
EOD Status:	12/6/2022 1:10 AM	By:	Cadence, Batch Job	ES

Patient as-of Visit

History as of 12/2/2022

Medical History as of 12/2/2022

Medical last reviewed by Brickman, Michael J., RN on 12/2/2022

Past Medical History

Diagnosis	Date	Comments	Source
COVID-19	—	—	Provider
Sinusitis	—	—	Provider
Ulcerative colitis (HCC)	—	—	Provider
Vertigo	—	—	Provider

Surgical History as of 12/2/2022

Surgical last reviewed by Brickman, Michael J., RN on 12/2/2022

Past Surgical History

Procedure	Laterality	Date	Comments	Source
FRACTURE SURGERY	—	2016	left hand	Provider
BRAIN SURGERY	—	—	—	Provider

Family History as of 12/2/2022

Family History as of 12/2/2022

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Patient as-of Visit (continued)

Brother

Relationship: Brother
Name: —
Status: Alive
Age: —
Genetic Sex: Male
Gender Identity: Male
Father: Patient's Father
Mother: Patient's Mother
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
No Known Problems		

Brother

Relationship: Brother
Name: —
Status: Alive
Age: —
Genetic Sex: Male
Gender Identity: Male
Father: Patient's Father
Mother: Patient's Mother
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
No Known Problems		

Father

Relationship: Father
Name: —
Status: Deceased
Death Age: —
Genetic Sex: Male
Gender Identity: Male
Father: —
Mother: —
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
Diabetes		

Maternal Grandmother

Relationship: Maternal Grandmother
Name: —
Status: —
Age: —
Genetic Sex: Female
Gender Identity: Female
Father: —
Mother: —

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Patient as-of Visit (continued)

Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
Diabetes		

Mother

Relationship: Mother
Name: —
Status: Alive
Age: —
Genetic Sex: Female
Gender Identity: Female
Father: —
Mother: Patient's Maternal Grandmother
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
Diabetes		
Hypertension		

Substance & Sexuality History as of 12/2/2022

Tobacco Use as of 12/2/2022

Tobacco Use last reviewed by Brickman, Michael J., RN on 12/2/2022

Smoking Status	Smoking Start Date	Last Attempt to Quit	Smoking Frequency
Every Day	—	—	

Smokeless Status	Smokeless Type	Smokeless Quit Date
Never	—	—

Source
Provider

Alcohol Use as of 12/2/2022

Alcohol Use last reviewed by Brickman, Michael J., RN on 12/2/2022

Alcohol Use	Drinks/Week	Alcohol/Week	Comments	Source
Not Currently		—	—	Provider

Drug Use as of 12/2/2022

Drug Use last reviewed by Brickman, Michael J., RN on 12/2/2022

Drug Use	Types	Frequency	Comments	Source
Not Currently	—	—	—	Provider

Sexual Activity as of 12/2/2022

Sexual Activity last reviewed by Brickman, Michael J., RN on 12/2/2022

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Patient as-of Visit (continued)

	Sexually Active	Birth Control	Partners	Comments	Source
	Defer	—	—	—	Provider

Socioeconomic History as of 12/2/2022

Socioeconomic as of 12/2/2022

	Marital Status	Spouse Name	Number of Children	Years Education	Education Level	Preferred Language	Ethnicity	Race	Source
	Single	—	—	—	—	English	Non-Hispanic	Black or African American	—

Social Documentation History as of 12/2/2022

Social Documentation last reviewed by Brickman, Michael J., RN on 12/2/2022
None

Immunizations as of 12/2/2022

Immunizations never marked as reviewed

Tdap *[last edited by Joyce, Melanie M., RN on 2/10/2021 1943]*

Administered by: Joyce, Melanie M., RN
Administered on: 2/10/2021 1942
Dose: 0.5 mL
Site: Left deltoid
Route: Intramuscular
NDC: 58160-842-43
CVX code: 115
VIS date: 4/1/20
Manufacturer: GlaxoSmithKline
Lot number: AB374
Expiration date: 11/21/2022

Questionnaire

Question	Answer
Have you ever had a serious reaction to any vaccine in the past?	—
Are you sick today with a moderate to severe illness (e.g. fever)	—
VIS given date	2/10/2021

Allergies as of 12/2/2022

Allergies last reviewed by Brickman, Michael J., RN on 12/2/2022 1500

OTHER *[last edited by Harris, David Michael, RN on 6/26/2022 1926]*

Reactions: Itching, Swelling
Severity: Medium
Noted on: 06/26/2022
Comments: Rye Bread

Problem List as of 12/2/2022

Problems last reviewed by Cook, Brandy La-Keysha, NP on 7/14/2022 0934

Alcohol use *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0951]*

Diagnosis: Alcohol use
Noted on: 07/14/2022
Chronic: No

Appendicitis *[last edited by Sallis, Milton, MD on 7/14/2022 0203]*

Diagnosis: Appendicitis
Noted on: 07/14/2022
Chronic: No

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Patient as-of Visit (continued)

Constipation *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Constipation	Noted on: 07/14/2022	Chronic: No

Contusion of chest *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Contusion of chest	Noted on: 06/26/2022	Chronic: No

Contusion of right calf *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Contusion of right calf	Noted on: 06/26/2022	Chronic: No

Contusion of right elbow *[last edited by Smith, Tavia N., NP on 6/26/2022 2326]*

Diagnosis: Contusion of right elbow	Noted on: 06/26/2022	Chronic: No

Gastroesophageal reflux disease without esophagitis *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Gastroesophageal reflux disease without esophagitis	Noted on: 07/14/2022	Chronic: No

Generalized abdominal pain *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Generalized abdominal pain	Noted on: 07/14/2022	Chronic: No

MVC (motor vehicle collision), initial encounter *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: MVC (motor vehicle collision), initial encounter	Noted on: 06/26/2022	Chronic: No

Nausea & vomiting *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Nausea & vomiting	Noted on: 07/14/2022	Chronic: No

Pansinusitis *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Pansinusitis	Noted on: 01/22/2021	Chronic: No

Primary hypertension *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Primary hypertension	Noted on: 07/14/2022	Chronic: No

Skin burn *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Skin burn	Noted on: 07/14/2022	Chronic: No

Sprain of shoulder, right *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Sprain of shoulder, right	Noted on: 06/26/2022	Chronic: No

Strain of lumbar region *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Strain of lumbar region	Noted on: 06/26/2022	Chronic: No

Tobacco abuse *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0951]*

Diagnosis: Tobacco abuse	Noted on: 07/14/2022	Chronic: No

Care Team as of 12/2/2022

Active

Name	Relationship	Specialty	Phone	Duration
NO, PHYSICIAN	PCP - General	—	999-999-9999	06/26/2022 - Present
NO, PHYSICIAN Comment: Merged	—	—	999-999-9999	04/20/2022 - Present
RAFIQ, REHAN Comment: Added from order Follow-up with established providerIn the next 7-10 days	Consulting Physician	Gastroenterology	314-953-8779	07/14/2022 - Present

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Patient as-of Visit (continued)

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Case 9882993

Surgery Information

General Information

Date: 12/2/2022 | Time: | Status: Posted
Location: CH INVASIVE | Room: | Service:
Patient class: | Case classification:

Diagnosis Information

No post-op diagnosis codes associated with the log.

Clinical Documentation

Case Tracking Events

Event	Time In
In Facility	1527

Event Tracking

No event timings documented

Patient Preparation

None

Counts by Panel

Panel 1

Type	Which?	Correct ?	X-Ray?	MD Notified?	Counted By	Verified By
Soft Goods	Initial					
Needles/Sharps	Initial					
Instruments	Initial					
Soft Goods	Final					
Needles/Sharps	Final					
Instruments	Final					

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology OpTime Log Detail

General Information

Date: 12/2/2022 | Time: | Status: Posted
Location: CH INVASIVE | Room: | Service:
Patient class: | Case classification:

The Asthma Action Plan has not been signed, or it has been modified since it was last signed. Please have a provider or nurse review and sign in order to print this document.

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology
Asthma Action Plan

12/02/2022 - Appointment in Christian Hospital Northwest HealthCare Imaging and Radiology (continued)

Coding Summary

Account Information

Hospital Account	Primary Payor	Affiliated Recurring Accounts	Combined from HAR
202771461 - [redacted]	UNITED HEALTHCARE [201]	None	None

Admission Information

Arrival Date/Time:	12/02/2022 1557	Admit Date/Time:	12/02/2022 1444	IP Adm. Date/Time:	
Admission Type:	Emergency	Point of Origin:	Non-health Care Facility Point Of Origin	Admit Category:	
Means of Arrival:	Car	Primary Service:	Emergency	Secondary Service:	
Transfer Source:		Service Area:		Unit:	
Admit Provider:	NW XR ED	Attending Provider:	Abbott, Robert Emmons, MD	Referring Provider:	King, Kimberly Janean, NP

Discharge Information

Discharge Date/Time	Discharge Disposition	Discharge Destination	Discharge Provider	Unit
12/02/2022 1712	Discharge To Home Or Self Care	None	None	Christian Hospital Northwest HealthCare Imaging and Radiology

Admission Diagnoses / Reasons for Visit (ICD-10-CM)

Code	Description	Comments
S39.012A	Strain of muscle, fascia and tendon of lower back, initial encounter	

Final Diagnoses (ICD-10-CM)

Code	Description	POA	CC	HAC	Affects DRG
S39.012A **[Principal]**	Strain of muscle, fascia and tendon of lower back, initial encounter				
S80.01XA	Contusion of right knee, initial encounter				
S29.012A	Strain of muscle and tendon of back wall of thorax, initial encounter				
Z86.16	Personal history of COVID-19				
Z83.3	Family history of diabetes mellitus				

External Causes of Injury (ICD-10-CM)

Code	Description	POA	CC	HAC	Affects DRG
V43.52XA	Car driver injured in collision with other type car in traffic accident, initial encounter				
Y92.410	Unspecified street and highway as the place of occurrence of the external cause				

Care and staffing during this admission was provided during a governmental declaration of emergency, and during a time in which surge operation plans were invoked to respond to the volume of admissions related to the COVID-19 pandemic. This process began on 8/6/21 and continues. Surge Documentation Standards implemented on 1/11/22 at 6 a.m. and ended 3/9/22 at 7 a.m.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

BJC HealthCare

ADMISSION FORM

Encounter #	MR#	Enc Start Date Time	Pt Location
202771461	[redacted]	12/2/2022-1527	CH NW ED

CSN#	LOC	Clin Svc	PAT	Enc Type
[redacted]		Emergency		Emergency

Adm Dx Code Desc	Inf Dis	Visitor Restriction

Confidential	Adm Type	Adm Source	Last Enc Date
No	Emergency	1	

Birth Date - Age	Race	Ethnicity	Maiden Name
[redacted]	Black or African American	Non-Hispanic	

Gender	Marital	Religion	Soc Sec #
Male	Single	CHRISTIAN	[redacted]

Preferred Language	Birth Place	Health Care Proxy Sts	Patient Living Will Status
English			

Patient Name Address, Phone	Employer Name, Address, Phone	Emplm Sts
[redacted] Home: [redacted] Work: Mobile: [redacted]	,	NOT EMPLOYED

Guarantor Name, Pt Rel, Address, Phone	Guarantor Empr, Address, Phone	Emplm Sts
[redacted] Self Phone: [redacted] Work:	,	NOT EMPLOYED

EMERGENCY CONTACTS

Name	Home Phone	Work Phone	Mobile Phone	Relationship	Lgl Grd
[redacted]	[redacted]		[redacted]	Mother	No

Insurance 1 Name, Address, Phone

UNITED HEALTHCARE/UHC CHOICE PLUS

,

877-842-3210

Verified?

Policy Number	Group Number
[redacted]	[redacted]

Referral/Authorization #

Eff. Date	Pre-Cert. Phone
12/1/2021	

Subscriber DOB
[redacted]

Subscriber Employer

Insurance 2 Name, Address, Phone

CENTENE/HOME STATE HEALTH P*

PO BOX 4050
FARMINGTON, MO 63640-3829

855-694-4663

Verified?

Policy Number	Group Number
[redacted]	

Referral/Authorization #

Eff. Date	Pre-Cert. Phone
12/2/2022	

Subscriber DOB
[redacted]

Subscriber Employer

Insurance 3 Name, Address, Phone

/

,

Verified?

Policy Number	Group Number

Referral/Authorization #

Eff. Date	Pre-Cert. Phone

Subscriber DOB

Subscriber Employer

Insurance 4 Name, Address, Phone

/

,

Verified?

Policy Number	Group Number

Referral/Authorization #

Eff. Date	Pre-Cert. Phone

Subscriber DOB

Subscriber Employer

Primary Physician	Referring Physician	Admitting Physician, ID	Attending Physician, ID
No, Physician 999-999-9999		No admitting provider for patient encounter.	No att. providers found

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Incident Date	Incident Type	Inc St/Prov	Incident Description / Location

FOR EMS ONLY:

Reason for Visit

Chief Complaint *[last edited by Brickman, Michael J., RN on 12/2/2022 1459]*

- Motor Vehicle Crash

Visit Diagnoses *[last edited by King, Kimberly Janean, NP on 12/2/2022 1659]*

Name	Is ED?
Motor vehicle accident, initial encounter (primary)	Yes
Strain of trapezius muscle, unspecified laterality, initial encounter	Yes
Strain of lumbar region, initial encounter	Yes
Contusion of right knee, initial encounter	Yes

Visit Information

Admission Information

Arrival Date/Time:	12/02/2022 1444	Admit Date/Time:	12/02/2022 1527	IP Adm. Date/Time:	
Admission Type:	Emergency	Point of Origin:	Non-health Care Facility Point Of Origin	Admit Category:	
Means of Arrival:	Car	Primary Service:	Emergency	Secondary Service:	N/A
Transfer Source:		Service Area:	BJC HEALTHCARE	Unit:	Christian Hospital Northwest Emergency Department
Admit Provider:		Attending Provider:	Abbott, Robert Emmons, MD	Referring Provider:	

ED Disposition

ED Disposition	Condition	User	Date/Time	Comment
Discharge	Stable	King, Kimberly Janean, NP	Fri Dec 2, 2022 4:58 PM	--

Discharge Information

Date/Time: 12/02/2022 1712	Disposition: Discharge To Home Or Self Care	Destination: —
Provider: —	Unit: Christian Hospital Northwest Emergency Department	

Follow-up Information

Follow up With	Specialties	Details	Why	Contact Info
YOUR PHYSICIAN or PHYSICIAN PROVIDED		Schedule an appointment as soon as possible for a visit	As needed, If symptoms worsen	
Scrivner, Katherine Anne, DO	Interventional Cardiology, Internal Medicine	Schedule an appointment as soon as possible for a visit	As needed, If symptoms worsen, To establish primary care	637 DUNN RD STE 170 Hazelwood MO 63042 314-838-5702

Treatment Team

Provider	Service	Role	Provider Team	Specialty	From	To
Abbott, Robert Emmons, MD	—	Attending Provider	—	Emergency Medicine	12/02/22 1654	12/02/22 1712
Hopper, Teresa	—	Registered Nurse	—	Emergency	12/02/22 1619	—

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Treatment Team (continued)

Provider	Service	Role	Provider Team	Specialty	From	To
L., RN King, Kimberly Janean, NP	—	Nurse Practitioner	—	Medicine Emergency Medicine	12/02/22 1531	—

Events

ED Arrival at 12/2/2022 1444

Unit: Christian Hospital Northwest Emergency Department
User: Lorino, Kenneth B.

Admission at 12/2/2022 1527

Unit: Christian Hospital Northwest Emergency Department — Room: CHNW PED24 — Bed: PED24
User: Brickman, Michael J., RN — Patient class: Emergency

Discharge at 12/2/2022 1712

Unit: Christian Hospital Northwest Emergency Department — Room: CHNW PED24 — Bed: PED24
User: Hopper, Teresa L., RN — Patient class: Emergency

Vitals

Date/Time	Temp	Pulse	Resp	BP	SpO2	Weight
12/02/22 1448	36.7 °C (98 °F)	79	18	112/68	97 %	77.1 kg (170 lb)

Current Medications

Medication List

ⓘ This report is for documentation purposes only. The patient should not follow medication instructions within. For accurate instructions regarding medications, the patient should instead consult their physician or after visit summary.

Prior To Admission

Medications last reviewed by Brickman, Michael J., RN on 12/2/2022 1501

ondansetron (ZOFRAN) 4 mg tablet

Instructions: Take 1 tablet (4 mg total) by mouth every 8 (eight) hours as needed for nausea or vomiting
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 20 tablet
Refill: No refills remaining

acetaminophen (TYLENOL) 325 mg tablet

Discontinued by: King, Kimberly Janean, NP — Discontinued on: 12/2/2022
Instructions: Take 2 tablets (650 mg total) by mouth every 6 (six) hours as needed for pain
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — End date: 12/2/2022
Quantity: 30 tablet — Refill: No refills remaining

bisacodyl EC (DULCOLAX EC) 5 mg EC tablet

Instructions: Take 2 tablets (10 mg total) by mouth daily as needed for constipation (If no results 24 hours after polyethylene glycol (MIRALAX). May give bisacodyl supp if not tolerating PO))
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 tablet
Refill: No refills remaining

montelukast (SINGULAIR) 10 mg tablet

Instructions: Take 1 tablet (10 mg total) by mouth nightly
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 tablet

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Current Medications (continued)

Refill: No refills remaining

polyethylene glycol (MIRALAX) 17 gram packet

Instructions: Take 1 packet (17 g total) by mouth daily as needed for constipation
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 30 packet
Refill: No refills remaining

famotidine (PEPCID) 20 mg tablet

Instructions: Take 1 tablet (20 mg total) by mouth 2 (two) times a day
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 60 tablet
Refill: No refills remaining

Discharge Medication List

Medications last reviewed by Brickman, Michael J., RN on 12/2/2022 1501

ondansetron (ZOFRAN) 4 mg tablet

Instructions: Take 1 tablet (4 mg total) by mouth every 8 (eight) hours as needed for nausea or vomiting
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 20 tablet
Refill: No refills remaining

bisacodyl EC (DULCOLAX EC) 5 mg EC tablet

Instructions: Take 2 tablets (10 mg total) by mouth daily as needed for constipation (If no results 24 hours after polyethylene glycol (MIRALAX). May give bisacodyl supp if not tolerating PO))
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 30 tablet
Refill: No refills remaining

montelukast (SINGULAIR) 10 mg tablet

Instructions: Take 1 tablet (10 mg total) by mouth nightly
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 30 tablet
Refill: No refills remaining

polyethylene glycol (MIRALAX) 17 gram packet

Instructions: Take 1 packet (17 g total) by mouth daily as needed for constipation
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 30 packet
Refill: No refills remaining

famotidine (PEPCID) 20 mg tablet

Instructions: Take 1 tablet (20 mg total) by mouth 2 (two) times a day
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Ordered on: 7/14/2022
Quantity: 60 tablet
Refill: No refills remaining

acetaminophen (TYLENOL) 500 mg tablet

Instructions: Take 1 tablet (500 mg total) by mouth every 6 (six) hours as needed for headaches or pain
Authorized by: King, Kimberly Janean, NP
Start date: 12/2/2022
Ordered on: 12/2/2022
Quantity: 30 tablet
Refill: No refills remaining

methocarbamoL (ROBAXIN) 500 mg tablet

Instructions: Take 1 tablet (500 mg total) by mouth 2 (two) times a day
Authorized by: King, Kimberly Janean, NP
Ordered on: 12/2/2022

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Current Medications (continued)

Start date: 12/2/2022 Quantity: 20 tablet
Refill: No refills remaining

naproxen (NAPROSYN) 500 mg tablet

Instructions: Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals
Authorized by: King, Kimberly Janean, NP Ordered on: 12/2/2022
Start date: 12/2/2022 Quantity: 10 tablet
Refill: No refills remaining

Stopped in Visit

None

Patient as-of Visit

History as of 12/2/2022

Medical History as of 12/2/2022

Medical last reviewed by Brickman, Michael J., RN on 12/2/2022

Past Medical History

Diagnosis	Date	Comments	Source
COVID-19	—	—	Provider
Sinusitis	—	—	Provider
Ulcerative colitis (HCC)	—	—	Provider
Vertigo	—	—	Provider

Surgical History as of 12/2/2022

Surgical last reviewed by Brickman, Michael J., RN on 12/2/2022

Past Surgical History

Procedure	Laterality	Date	Comments	Source
FRACTURE SURGERY	—	2016	left hand	Provider
BRAIN SURGERY	—	—	—	Provider

Family History as of 12/2/2022

Family History as of 12/2/2022

Brother

Relationship: Brother
Name: —
Status: Alive
Age: —
Genetic Sex: Male
Gender Identity: Male
Father: Patient's Father
Mother: Patient's Mother
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
No Known Problems		

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Patient as-of Visit (continued)

Brother

Relationship: Brother
Name: —
Status: Alive
Age: —
Genetic Sex: Male
Gender Identity: Male
Father: Patient's Father
Mother: Patient's Mother
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
No Known Problems		

Father

Relationship: Father
Name: —
Status: Deceased
Death Age: —
Genetic Sex: Male
Gender Identity: Male
Father: —
Mother: —
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
Diabetes		

Maternal Grandmother

Relationship: Maternal Grandmother
Name: —
Status: —
Age: —
Genetic Sex: Female
Gender Identity: Female
Father: —
Mother: —
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
Diabetes		

Mother

Relationship: Mother
Name: —
Status: Alive
Age: —
Genetic Sex: Female
Gender Identity: Female
Father: —

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Patient as-of Visit (continued)

Mother: Patient's Maternal Grandmother
Linked with: —
Comment: —
Fertility Status: —
Fertility Comment: —

Condition	Age of Onset	Comment
Diabetes		
Hypertension		

Substance & Sexuality History as of 12/2/2022

Tobacco Use as of 12/2/2022

Tobacco Use last reviewed by Brickman, Michael J., RN on 12/2/2022

Smoking Status	Smoking Start Date	Last Attempt to Quit	Smoking Frequency
Every Day	—	—	

Smokeless Status	Smokeless Type	Smokeless Quit Date
Never	—	—

Source
Provider

Alcohol Use as of 12/2/2022

Alcohol Use last reviewed by Brickman, Michael J., RN on 12/2/2022

Alcohol Use	Drinks/Week	Alcohol/Week	Comments	Source
Not Currently		—	—	Provider

Drug Use as of 12/2/2022

Drug Use last reviewed by Brickman, Michael J., RN on 12/2/2022

Drug Use	Types	Frequency	Comments	Source
Not Currently	—	—	—	Provider

Sexual Activity as of 12/2/2022

Sexual Activity last reviewed by Brickman, Michael J., RN on 12/2/2022

Sexually Active	Birth Control	Partners	Comments	Source
Defer	—	—	—	Provider

Socioeconomic History as of 12/2/2022

Socioeconomic as of 12/2/2022

Marital Status	Spouse Name	Number of Children	Years Education	Education Level	Preferred Language	Ethnicity	Race	Source
Single	—	—	—	—	English	Non-Hispanic	Black or African American	—

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Patient as-of Visit (continued)

Social Documentation History as of 12/2/2022

Social Documentation last reviewed by Brickman, Michael J., RN on 12/2/2022
None

Immunizations as of 12/2/2022

Immunizations never marked as reviewed

Tdap *[last edited by Joyce, Melanie M., RN on 2/10/2021 1943]*

Administered by: Joyce, Melanie M., RN	Administered on: 2/10/2021 1942	Dose: 0.5 mL
Site: Left deltoid	Route: Intramuscular	NDC: 58160-842-43
CVX code: 115	VIS date: 4/1/20	
Manufacturer: GlaxoSmithKline	Lot number: AB374	Expiration date: 11/21/2022

Questionnaire

Question	Answer
Have you ever had a serious reaction to any vaccine in the past?	—
Are you sick today with a moderate to severe illness (e.g. fever)	—
VIS given date	2/10/2021

Allergies as of 12/2/2022

Allergies last reviewed by Brickman, Michael J., RN on 12/2/2022 1500

OTHER *[last edited by Harris, David Michael, RN on 6/26/2022 1926]*

Reactions: Itching, Swelling	Severity: Medium
Noted on: 06/26/2022	
Comments: Rye Bread	

Problem List as of 12/2/2022

Problems last reviewed by Cook, Brandy La-Keysha, NP on 7/14/2022 0934

Alcohol use *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0951]*

Diagnosis: Alcohol use	Noted on: 07/14/2022	Chronic: No

Appendicitis *[last edited by Sallis, Milton, MD on 7/14/2022 0203]*

Diagnosis: Appendicitis	Noted on: 07/14/2022	Chronic: No

Constipation *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Constipation	Noted on: 07/14/2022	Chronic: No

Contusion of chest *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Contusion of chest	Noted on: 06/26/2022	Chronic: No

Contusion of right calf *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Contusion of right calf	Noted on: 06/26/2022	Chronic: No

Contusion of right elbow *[last edited by Smith, Tavia N., NP on 6/26/2022 2326]*

Diagnosis: Contusion of right elbow	Noted on: 06/26/2022	Chronic: No

Gastroesophageal reflux disease without esophagitis *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Patient as-of Visit (continued)

Diagnosis: Gastroesophageal reflux disease without esophagitis — Noted on: 07/14/2022 — Chronic: No

Generalized abdominal pain *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Generalized abdominal pain — Noted on: 07/14/2022 — Chronic: No

MVC (motor vehicle collision), initial encounter *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: MVC (motor vehicle collision), initial encounter — Noted on: 06/26/2022 — Chronic: No

Nausea & vomiting *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Nausea & vomiting — Noted on: 07/14/2022 — Chronic: No

Pansinusitis *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Pansinusitis — Noted on: 01/22/2021 — Chronic: No

Primary hypertension *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Primary hypertension — Noted on: 07/14/2022 — Chronic: No

Skin burn *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0933]*

Diagnosis: Skin burn — Noted on: 07/14/2022 — Chronic: No

Sprain of shoulder, right *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Sprain of shoulder, right — Noted on: 06/26/2022 — Chronic: No

Strain of lumbar region *[last edited by Smith, Tavia N., NP on 6/26/2022 2325]*

Diagnosis: Strain of lumbar region — Noted on: 06/26/2022 — Chronic: No

Tobacco abuse *[last edited by Cook, Brandy La-Keysha, NP on 7/14/2022 0951]*

Diagnosis: Tobacco abuse — Noted on: 07/14/2022 — Chronic: No

Care Team as of 12/2/2022

Active

Name	Relationship	Specialty	Phone	Duration
NO, PHYSICIAN	PCP - General	—	999-999-9999	06/26/2022 - Present
NO, PHYSICIAN	—	—	999-999-9999	04/20/2022 - Present
Comment: Merged				
RAFIQ, REHAN	Consulting Physician	Gastroenterology	314-953-8779	07/14/2022 - Present
Comment: Added from order Follow-up with established providerIn the next 7-10 days				

Medication List

Medication List

This report is for documentation purposes only. The patient should not follow medication instructions within. For accurate instructions regarding medications, the patient should instead consult their physician or after visit summary.

Prior To Admission

Medications last reviewed by Brickman, Michael J., RN on 12/2/2022 1501

ondansetron (ZOFRAN) 4 mg tablet

Instructions: Take 1 tablet (4 mg total) by mouth every 8 (eight) hours as needed for nausea or vomiting
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 20 tablet

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Medication List (continued)

Refill: No refills remaining

acetaminophen (TYLENOL) 325 mg tablet

Discontinued by: King, Kimberly Janean, NP — Discontinued on: 12/2/2022
Instructions: Take 2 tablets (650 mg total) by mouth every 6 (six) hours as needed for pain
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — End date: 12/2/2022
Quantity: 30 tablet — Refill: No refills remaining

bisacodyl EC (DULCOLAX EC) 5 mg EC tablet

Instructions: Take 2 tablets (10 mg total) by mouth daily as needed for constipation (If no results 24 hours after polyethylene glycol (MIRALAX). May give bisacodyl supp if not tolerating PO))
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 tablet
Refill: No refills remaining

montelukast (SINGULAIR) 10 mg tablet

Instructions: Take 1 tablet (10 mg total) by mouth nightly
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 tablet
Refill: No refills remaining

polyethylene glycol (MIRALAX) 17 gram packet

Instructions: Take 1 packet (17 g total) by mouth daily as needed for constipation
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 packet
Refill: No refills remaining

famotidine (PEPCID) 20 mg tablet

Instructions: Take 1 tablet (20 mg total) by mouth 2 (two) times a day
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 60 tablet
Refill: No refills remaining

Discharge Medication List

Medications last reviewed by Brickman, Michael J., RN on 12/2/2022 1501

ondansetron (ZOFRAN) 4 mg tablet

Instructions: Take 1 tablet (4 mg total) by mouth every 8 (eight) hours as needed for nausea or vomiting
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 20 tablet
Refill: No refills remaining

bisacodyl EC (DULCOLAX EC) 5 mg EC tablet

Instructions: Take 2 tablets (10 mg total) by mouth daily as needed for constipation (If no results 24 hours after polyethylene glycol (MIRALAX). May give bisacodyl supp if not tolerating PO))
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 tablet
Refill: No refills remaining

montelukast (SINGULAIR) 10 mg tablet

Instructions: Take 1 tablet (10 mg total) by mouth nightly
Authorized by: Khoukaz, Ghassan Hikmat, MD — Ordered on: 7/14/2022
Start date: 7/14/2022 — Quantity: 30 tablet
Refill: No refills remaining

polyethylene glycol (MIRALAX) 17 gram packet

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Medication List (continued)

Instructions: Take 1 packet (17 g total) by mouth daily as needed for constipation
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Refill: No refills remaining
Ordered on: 7/14/2022
Quantity: 30 packet

famotidine (PEPCID) 20 mg tablet

Instructions: Take 1 tablet (20 mg total) by mouth 2 (two) times a day
Authorized by: Khoukaz, Ghassan Hikmat, MD
Start date: 7/14/2022
Refill: No refills remaining
Ordered on: 7/14/2022
Quantity: 60 tablet

acetaminophen (TYLENOL) 500 mg tablet

Instructions: Take 1 tablet (500 mg total) by mouth every 6 (six) hours as needed for headaches or pain
Authorized by: King, Kimberly Janean, NP
Start date: 12/2/2022
Refill: No refills remaining
Ordered on: 12/2/2022
Quantity: 30 tablet

methocarbamoL (ROBAXIN) 500 mg tablet

Instructions: Take 1 tablet (500 mg total) by mouth 2 (two) times a day
Authorized by: King, Kimberly Janean, NP
Start date: 12/2/2022
Refill: No refills remaining
Ordered on: 12/2/2022
Quantity: 20 tablet

naproxen (NAPROSYN) 500 mg tablet

Instructions: Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals
Authorized by: King, Kimberly Janean, NP
Start date: 12/2/2022
Refill: No refills remaining
Ordered on: 12/2/2022
Quantity: 10 tablet

Stopped in Visit

None

ED Provider Note

ED Provider Notes by King, Kimberly Janean, NP at 12/2/2022 1655

Author: King, Kimberly Janean, NP
Filed: 12/2/2022 5:00 PM
Editor: King, Kimberly Janean, NP (Nurse Practitioner)
Service: Emergency Medicine
Date of Service: 12/2/2022 4:55 PM
Author Type: Nurse Practitioner
Status: Signed
Cosigner: Abbott, Robert Emmons, MD at 12/2/2022 6:32 PM

HPI

Chief Complaint

Patient presents with

- Motor Vehicle Crash[KK.1T]

40-year-old male with a past medical history that includes COVID-19, sinusitis, ulcerative colitis, vertigo, presents to the emergency room with complaints of pain to the left side of his body, right knee pain, and pain across his upper and lower back that he reports started after he was involved in a motor vehicle accident last night. Was the restrained driver, the vehicle he was traveling in was struck in the front driver's side, there was full airbag deployment. Denies hitting his head, no LOC, no nausea or vomiting, no numbness, no tingling, no weakness, no wounds, no chest pain, no shortness of breath, no abdominal pain. He is not taken anything for his symptoms. There are no other complaints

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Provider Note (continued)

expressed at this time.[KK.2M]

Cortez D Parker[KK.1T], a[KK.3T] 40 y.o. male[KK.1T]
presents to the ED[KK.3T]

Chief Complaint

Patient presents with

- Motor Vehicle Crash[KK.1T]

Triage nursing note reviewed and confirmed

PMH:[KK.3T]

Past Medical History:

Diagnosis	Date
• COVID-19	
• Sinusitis	
• Ulcerative colitis (HCC)	
• Vertigo	

Patient History:

Past Medical History:

Diagnosis	Date
• COVID-19	
• Sinusitis	
• Ulcerative colitis (HCC)	
• Vertigo	

Past Surgical History:

Procedure	Laterality	Date
• BRAIN SURGERY		
• FRACTURE SURGERY		2016
left hand		

Family History

Problem	Relation	Age of Onset
• Hypertension	Mother	
• Diabetes	Mother	
• Diabetes	Father	
• No Known Problems	Brother	
• No Known Problems	Brother	
• Diabetes	Maternal Grandmother	

Social History

Tobacco Use	
• Smoking status:	Every Day

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Provider Note (continued)

- Smokeless tobacco: Never

Substance and Sexual Activity

- Drug use: Not Currently
- Sexual activity: Defer

Alcohol Use: Heavy Drinker

- Frequency of Alcohol Consumption: 2-3 times a week
- Average Number of Drinks: 1 or 2
- Frequency of Binge Drinking: Less than monthly[KK.1T]

Review of Systems

Review of Systems
Constitutional: Negative for[KK.3T] activity change[KK.2M],[KK.3T] chills[KK.2M] and[KK.3T] fever[KK.2M].
HENT: Negative for[KK.3T] facial swelling[KK.2M].
Eyes: Negative for[KK.3T] visual disturbance[KK.2M].
Respiratory: Negative for[KK.3T] cough[KK.2M],[KK.3T] chest tightness[KK.2M] and[KK.3T] shortness of breath[KK.2M].
Cardiovascular: Negative for[KK.3T] chest pain[KK.2M] and[KK.3T] palpitations[KK.2M].
Gastrointestinal: Negative for[KK.3T] abdominal pain[KK.2M],[KK.3T] diarrhea[KK.2M],[KK.3T] nausea[KK.2M] and[KK.3T] vomiting[KK.2M].
Genitourinary: Negative for[KK.3T] dysuria[KK.2M] and[KK.3T] hematuria[KK.2M].
Musculoskeletal: Positive for[KK.3T] arthralgias **(right knee, left-sided body)**[KK.2M] and[KK.3T] back pain **(upper and lower)**[KK.2M]. Negative for[KK.3T] joint swelling[KK.2M],[KK.3T] myalgias[KK.2M] and[KK.3T] neck pain[KK.2M].
Skin: Negative for[KK.3T] color change[KK.2M],[KK.3T] rash[KK.2M] and[KK.3T] wound[KK.2M].
Neurological: Negative for[KK.3T] dizziness[KK.2M],[KK.3T] syncope[KK.2M],[KK.3T] weakness[KK.2M],[KK.3T] light-headedness[KK.2M],[KK.3T] numbness[KK.2M] and[KK.3T] headaches[KK.2M].[KK.3T]
All other systems reviewed and are negative[KK.2M].[KK.3T]

Physical Exam

ED Triage Vitals [12/02/22 1448]

Temp	Pulse	Resp	BP	SpO2
36.7 °C (98 °F)	**79**	**18**	**112/68**	**97 %**

Temp src	Heart Rate Source	Patient Position	BP Location	FiO2 (%)
Tympanic	**Monitor**	**--**	**Right arm**	**--**

Height	Height Method	Weight	Weight Method
1.88 m (6' 2")	**Stated**	**77.1 kg (170 lb)**	**Stated**[KK.1T]

Physical Exam[KK.3T]
Vitals[KK.2M] and[KK.3T] nursing note[KK.2M] reviewed.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Provider Note (continued)

Constitutional:
Appearance: He is[KK.3T] well-developed[KK.2M].
HENT:
Head:[KK.3T] Normocephalic[KK.2M] and[KK.3T] atraumatic[KK.2M].
Nose:[KK.3T] Nose normal[KK.2M].
Mouth/Throat:
Mouth: Mucous membranes are[KK.3T] moist[KK.2M].
Pharynx:[KK.3T] Oropharynx is clear[KK.2M].
Eyes:
Conjunctiva/sclera:[KK.3T] Conjunctivae normal[KK.2M].
Cardiovascular:
Rate and Rhythm:[KK.3T] Normal rate[KK.2M] and[KK.3T] regular rhythm[KK.2M].
Pulses:[KK.3T] Normal pulses[KK.2M].
Heart sounds:[KK.3T] Normal heart sounds[KK.2M].
Pulmonary:
Effort: Pulmonary effort is[KK.3T] normal[KK.2M].
Breath sounds: Normal[KK.3T] breath sounds[KK.2M].
Abdominal:
General: Bowel sounds are[KK.3T] normal[KK.2M].
Palpations: Abdomen is[KK.3T] soft[KK.2M].
Tenderness: There is[KK.3T] no abdominal tenderness[KK.2M]. There is no[KK.3T] guarding[KK.2M].
Musculoskeletal:
General:[KK.3T] Normal range of motion[KK.2M].
Cervical back:[KK.3T] Normal range of motion[KK.2M] and[KK.3T] neck supple[KK.2M].[KK.3T] Tenderness **(bilateral trapezius with palpation)**[KK.2M] present. No[KK.3T] bony tenderness[KK.2M].[KK.3T] Muscular tenderness **(left-sided paraspinal with palpation)**[KK.2M] present. No[KK.3T] spinous process tenderness[KK.2M].[KK.3T] Normal range of motion[KK.2M].
Thoracic back: No[KK.3T] tenderness[KK.2M] or[KK.3T] bony tenderness[KK.2M].[KK.3T] Normal range of motion[KK.2M].
Lumbar back:[KK.3T] Tenderness **(bilateral paraspinal with palpation)**[KK.2M] present. No[KK.3T] bony tenderness[KK.2M].[KK.3T] Normal range of motion[KK.2M].
Right knee: No[KK.3T] swelling[KK.2M],[KK.3T] deformity[KK.2M],[KK.3T] effusion[KK.2M],[KK.3T] erythema[KK.2M],[KK.3T] ecchymosis[KK.2M] or[KK.3T] lacerations[KK.2M].[KK.3T] Normal range of motion[KK.2M].[KK.3T] Tenderness **(generalized with palpation)**[KK.2M] present.[KK.3T] Normal alignment[KK.2M].[KK.3T] Normal pulse[KK.2M].
Skin:
General: Skin is[KK.3T] warm[KK.2M] and[KK.3T] dry[KK.2M].
Capillary Refill: Capillary refill takes[KK.3T] less than 2 seconds[KK.2M].
Findings: No[KK.3T] bruising[KK.2M],[KK.3T] erythema[KK.2M] or[KK.3T] rash[KK.2M].
Neurological:
Mental Status: He is[KK.3T] alert[KK.2M] and[KK.3T] oriented to person, place, and time[KK.2M].[KK.3T]

Procedures[KK.3M]

MDM

Labs Reviewed - No data to display

XR Knee Right 4 or More Views
Final Result
FINDINGS/IMPRESSION:

There is no sign of any acute osseous or articular abnormality. There

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Provider Note (continued)

are no specific soft tissue abnormalities.

Electronically signed by: Paul Hargan, M.D.

XR Spine Lumbar 4 or More Views
Final Result

UNREMARKABLE LUMBAR SPINE STUDY

Electronically signed by: Paul Hargan, M.D.

BP 112/68 (BP Location: Right arm) | Pulse 79 | Temp 36.7 °C (98 °F) (Tympanic) | Resp 18 | Ht 188 cm (6' 2") | Wt 77.1 kg (170 lb) | SpO2 97% | BMI 21.83 kg/m^2[KK.1T]

MDM

Number of Diagnoses or Management Options[KK.3T]
Contusion of right knee, initial encounter
Motor vehicle accident, initial encounter
Strain of lumbar region, initial encounter
Strain of trapezius muscle, unspecified laterality, initial encounter[KK.2M]
Diagnosis management comments:[KK.3T] In my medical decision making the following differential diagnoses were considered before arriving at final diagnosis and many were either ruled out or appeared unlikely:

MVA (or Trauma, Fall or Assault etc... injury):
Abdominal Injury, Abrasions / Contusions, Lower Extremity Injury, Head / Facial Injury, Chest Injury Upper Extremity Injury, Neck / Spinal Injury, Normal Exam[KK.2M]

Amount and/or Complexity of Data Reviewed
Tests in the radiology section of CPT®:[KK.3T] reviewed[KK.2M]

Risk of Complications, Morbidity, and/or Mortality
Presenting problems:[KK.3T] moderate[KK.2M]
Diagnostic procedures:[KK.3T] low[KK.2M]
Management options:[KK.3T] moderate[KK.2M]

Patient Progress
Patient progress:[KK.3T] stable[KK.2M]

5:00 PM[KK.1T] Rechecked[KK.2T] [redacted][KK.1T] is resting comfortably and feeling better. I discussed the results of diagnostic studies, my clinical impression, and the plan for further treatment with the[KK.2T] [redacted][KK.1T]. Patient agrees with plan and discharge at this time, all questions addressed.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Provider Note (continued)

Pt is medically stable for discharge at this time.

I have given[KK.2T] [redacted] r[KK.1T] instructions regarding the diagnosis, expectations, follow up, and return precautions. I explained to the patient that emergent conditions may arise and to return to the ER for new, worsening, or any persistent conditions. I've explained the importance of following up with the Primary Care Physician-(or the referral physician listed below) as instructed. The patient verbalized understanding of the discharge instructions.

New Medications:[KK.2T]

New Prescriptions

ACETAMINOPHEN (TYLENOL) 500 MG TABLET	Take 1 tablet (500 mg total) by mouth every 6 (six) hours as needed for headaches or pain
METHOCARBAMOL (ROBAXIN) 500 MG TABLET	Take 1 tablet (500 mg total) by mouth 2 (two) times a day
NAPROXEN (NAPROSYN) 500 MG TABLET	Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals[KK.1T]

I have advised the patient to follow-up with:[KK.2T]
YOUR PHYSICIAN or PHYSICIAN PROVIDED

Schedule an appointment as soon as possible for a visit
As needed, If symptoms worsen

Scrivner, Katherine Anne, DO
637 DUNN RD STE 170
Hazelwood MO 63042
314-838-5702

Schedule an appointment as soon as possible for a visit
As needed, If symptoms worsen, To establish primary care[KK.1T]

DIAGNOSIS:[KK.2T]

1. **Motor vehicle accident, initial encounter**
2. Strain of trapezius muscle, unspecified laterality, initial encounter
3. Strain of lumbar region, initial encounter
4. Contusion of right knee, initial encounter[KK.1T]

Disposition: Discharged[KK.2T]

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Provider Note (continued)

Impression:[KK.3T]
Motor vehicle accident, initial encounter

Strain of trapezius muscle, unspecified laterality, initial encounter

Strain of lumbar region, initial encounter

Contusion of right knee, initial encounter

King, Kimberly Janean, NP
12/02/22 1700
[KK.1T]

Electronically signed by King, Kimberly Janean, NP at 12/2/2022 5:00 PM
Electronically signed by Abbott, Robert Emmons, MD at 12/2/2022 6:32 PM

Attribution Key

KK.1 - King, Kimberly Janean, NP on 12/2/2022 5:00 PM
KK.2 - King, Kimberly Janean, NP on 12/2/2022 4:58 PM
KK.3 - King, Kimberly Janean, NP on 12/2/2022 4:55 PM
M - Manual, T - Template

ED Notes

ED Triage Notes by Brickman, Michael J., RN at 12/2/2022 1458

Author: Brickman, Michael J., RN | Service: — | Author Type: Registered Nurse
Filed: 12/2/2022 2:59 PM | Date of Service: 12/2/2022 2:58 PM | Status: Signed
Editor: Brickman, Michael J., RN (Registered Nurse)

Pt was restrained driver of an MVC,+airbag, hit head, -loc. Pt c/o L shoulder, arm and side pain. R knee pain and back pain.[MB.1M]

Electronically signed by Brickman, Michael J., RN at 12/2/2022 2:59 PM

Attribution Key

MB.1 - Brickman, Michael J., RN on 12/2/2022 2:58 PM
M - Manual

ED Care Timeline

Patient Care Timeline (12/2/2022 14:44 to 12/2/2022 17:12)

	12/2/2022	Event	Details	User
	14:44	**Patient arrived in ED**		Lorino, Kenneth B.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

14:44	**Patient arrived in ED**		Lorino, Kenneth B.
14:44:30	**Emergency encounter created**		Lorino, Kenneth B.
14:44:43	**Arrival Complaint**	MVC	
14:48	**Vital Signs**	**Vital Signs** Temp: 36.7 °C (98 °F) Temp src: Tympanic Pulse: 79 Heart Rate Source: Monitor Resp: 18 Resp Source: Visual BP: 112/68 BP Location: Right arm BP Method: Automatic SpO2: 97 % **Oxygen Therapy** SpO2: 97 % **Height and Weight** Height: 188 cm (6' 2") Height Method: Stated Weight: 77.1 kg (170 lb) Weight Method: Stated **Vitals Timer** Restart Vitals Timer: Yes	Donaldson, Kaviona
14:48	**Anthropometrics**	**Anthropometrics** Weight Change: -9.48 kg (-20.91 lbs)	Donaldson, Kaviona

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

14:48	**Custom Formula Data**	**Measurements** Total Weight Change Percent: 2222 Percent Weight Change Since Preop: 77.09 kgs Initial Excess Weight: -86.18 kgs Percent of IBW: 3156.18 Percent EBW (kg): 2717.56 kg EBW (lbs): 2708.13 lbs **Vital Signs** BMI (Calculated): 21.8 **Height and Weight** Weight in (lb) to have BMI = 25: 194.3 **Anthropometrics** Admission Weight : 77.1 kg **Relevant Labs and Vitals** Temp (in Celsius): 36.7 **Adult IBW/VT Calculations** IBW/kg (Calculated) : 82.2 Low Range Vt 6mL/kg : 493.2 mL/kg Adult Moderate Range Vt 8mL/kg : 657.6 mL/kg Adult High Range Vt 10mL/kg: 822 mL/kg **Other flowsheet entries** BMI (Calculated): 21.9 IBW in lbs (Bariatric): 190 Weight Change Since Last Visit: 77.09 kg IBW in kg (Bariatric): 86.18 Initial Rate: 12 Initial Rate: : 7.5 Initial Rate: 10 IBW/kg (Calculated) Male: 82.2 kg IBW/kg (Calculated) : 86.2 kg :: 12 :: 10 BSA (Calculated - sq m): 2 sq meters Admission Weight: 77.111	Donaldson, Kaviona
14:58:31	**ED Triage Notes**	Pt was restrained driver of an MVC,+airbag, hit head, -loc. Pt c/o L shoulder, arm and side pain. R knee pain and back pain.	Brickman, Michael J., RN
14:59	**Stroke Screening Tool**	**FAST** Facial Droop: Normal Arm Drift: Normal Speech: Normal **Assess for Posterior Circulation Stroke** Assess Horizontal Gaze: Normal Vison: Normal Loss of balance, stumbling or gait change: Normal Perform Finger to Nose test: Normal	Brickman, Michael J., RN
14:59	**Primary Assessment**	**Airway** Airway: Patent **Breathing** Breathing: Adequate **Circulation** Circulation : Adequate **Disability** Disability: Alert	Brickman, Michael J., RN

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

14:59	**Travel and Exposure Screening**	**Travel Screening** Have you traveled outside the U.S. in the last 6 months?: No **Exposure Screening** Have you been exposed to anyone who is sick in the last 30 days?: No **Infectious Disease Screening** Are you having any of the following:: None	Brickman, Michael J., RN
14:59	**Is patient diabetic**	**Is the patient diabetic** Is patient diabetic: No	Brickman, Michael J., RN
14:59	**Screen for infection**	**Infection Screen** Does the patient have an infection, or do you suspect they may have an infection: No Does the patient have risk factors for infection: No	Brickman, Michael J., RN
14:59:29	**Trigger for Triage Start**		Brickman, Michael J., RN
14:59:29	**Triage Started**		Brickman, Michael J., RN
14:59:29	**Chief Complaints Updated**	**Motor Vehicle Crash**	Brickman, Michael J., RN
15:00	**Acuity/Vertical**	**Acuity/Vertical** Patient Acuity: 4 Vertical: Vertical	Brickman, Michael J., RN
15:00	**Pain Assessment**	**Pain Assessment** Pain Assessment: 0-10 Pain Score: 8	Brickman, Michael J., RN
15:00	**Elopement Precautions**	**Elopement Risk Assessment** Risk/Reason for Elopement: No risk	Brickman, Michael J., RN
15:00	**Abuse Assessment**	**Abuse Assessment: Conduct in private to create environment of safety and confidentiality.** Have you ever been in or are you currently in a harmful physical or emotional relationship or is someone making you feel afraid or unsafe?: Denies Do you have someplace safe to go at discharge?: Yes Who answered abuse screen question?: Patient Who was in the room when abuse screen was answered?: Patient	Brickman, Michael J., RN
15:00	**Fall Risk Assessment (MEDFRAT)**	**Fall Risk Assessment Tool - MEDFRAT** History of falling in last 3 months, including since admission: No Prior Fall Event (Autopopulated from EMR): None found Confusion or disorientation: No Intoxicated or sedated: No Impaired gait: No Mobility assist device used: No Altered elimination: No Fall risk score: (1-2 low risk), (3-4 moderate risk), (5 or more high risk): 0	Brickman, Michael J., RN
15:00	**Pain Assessment**	**Pain Assessment Timer** Restart Pain Assessment Timer: Yes	Brickman, Michael J., RN

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

15:00	**C-SSRS NEW**	**Encounter Related Suicide Attempt/Behavior: Whenever possible, conduct screening in private to create a confidential environment.** Is this encounter related to a suicidal attempt/behavior?: No Information obtained from: Patient **Columbia Suicide Severity Rating Scale (Recent Screener)** 1. In the past month, have you wished you were dead or that you could go to sleep and not wake up?: No 2. In the past month, have you actually had any thoughts of killing yourself?: No 6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?: No	Brickman, Michael J., RN
15:00	**Custom Formula Data**	**Columbia Suicide Severity Rating Scale (Recent Screener)** Suicide Risk Level: No risk level	Brickman, Michael J., RN
15:00:05	**Vertical Patient**		Brickman, Michael J., RN
15:00:05	**Triage Completed**		Brickman, Michael J., RN
15:00:15	**Allergies Reviewed**		Brickman, Michael J., RN
15:00:24	**History Reviewed**	Sections Reviewed: Medical, Surgical, Tobacco, Alcohol, Drug Use, Sexual Activity, Custom, Social Documentation, Family	Brickman, Michael J., RN
15:01	**Tetanus/Immunization Status**	**Tetanus Up to Date** Is the patient's tetanus up to date?: Yes **Immunizations Up to Date** Are the patient's immunizations up to date?: Yes	Brickman, Michael J., RN
15:01:07	**Home Medications Reviewed**		Brickman, Michael J., RN
15:02	**Secondary Complete**		Brickman, Michael J., RN
15:02	**Secondary Assessment Complete**	**Secondary Assessment** Secondary Complete: Secondary Complete	Brickman, Michael J., RN
15:02	**HIV Screening**	**HIV Screening** HIV Screen Offered: Patient refused HIV screening Reason screening refused: No reason stated	Brickman, Michael J., RN
15:02	**Opioid Screening Tool**	**Opioid Screening Tool - In the Past 3 Months** Have you regularly used prescription pain medication like Percocet or Norco for non-medical reasons? : No Have you used illegal opioids such as heroin or fentanyl: No	Brickman, Michael J., RN
15:19:25	**Registration Completed**		Jones, Loaunia N.
15:27:27	**Patient roomed in ED**	To room CHNW PED24	Brickman, Michael J., RN
15:31:08	**Assign Mid-level**	King, Kimberly Janean, NP assigned as Nurse Practitioner	King, Kimberly Janean, NP
15:31:08	**Assign Physician**		King, Kimberly Janean, NP
15:31:08	**Assign First Provider**		King, Kimberly Janean, NP
15:31:13	**First Provider Evaluation of Patient**		King, Kimberly Janean, NP
15:40	**Early Detection of Sepsis**	**Other flowsheet entries** Sepsis Score: 1.13	Background User, Cogito

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

15:50:50	**Orders Placed**	Medications - ibuprofen (ADVIL,MOTRIN) tablet 800 mg; acetaminophen (TYLENOL) tablet 650 mg Imaging - XR Knee Right 4 or More Views; XR Spine Lumbar 4 or More Views	King, Kimberly Janean, NP
15:50:52	**XR Ordered**	XR SPINE LUMBAR 4 OR MORE VIEWS, XR KNEE RIGHT 4 OR MORE VIEWS	King, Kimberly Janean, NP
15:50:52	**Imaging Exam Ordered**		King, Kimberly Janean, NP
15:56:27	**Orders Acknowledged**	New - XR Knee Right 4 or More Views; XR Spine Lumbar 4 or More Views; ibuprofen (ADVIL,MOTRIN) tablet 800 mg; acetaminophen (TYLENOL) tablet 650 mg	Hopper, Teresa L., RN
15:57:57	**Imaging Exam Started**	XR Knee Right 4 or More Views	Dunham, Carrie An M, RT
15:57:57	**Imaging Exam Started**	XR Spine Lumbar 4 or More Views	Dunham, Carrie An M, RT
15:58	**Medication Given**	ibuprofen (ADVIL,MOTRIN) tablet 800 mg - Dose: **800 mg** ; Route: **oral** ; Scheduled Time: **1551**	Hopper, Teresa L., RN
15:58	**Medication Given**	acetaminophen (TYLENOL) tablet 650 mg - Dose: **650 mg** ; Route: **oral** ; Scheduled Time: **1551**	Hopper, Teresa L., RN
15:59	**Peripheral IV 07/13/22 22 G Right Antecubital Removed**	Removal Date/Time: 12/02/22 1559 Placement Date/Time: 07/13/22 2200 Type: Angiocath Size Gauge: 22 G Location Orientation: Right Location: Antecubital Site Prep: Chlorhexidine Technique: Anatomical landmarks Inserted by: Christian Insertio...	Hopper, Teresa L., RN
16:00	**Early Detection of Sepsis**	**Other flowsheet entries** Sepsis Score: 1.07	Background User, Cogito
16:19:22	**Assign Nurse**	Hopper, Teresa L., RN assigned as Registered Nurse	Hopper, Teresa L., RN
16:20	**Early Detection of Sepsis**	**Other flowsheet entries** Sepsis Score: 1.07	Background User, Cogito
16:23:22	**Imaging Exam Ended**	XR Knee Right 4 or More Views	Dunham, Carrie An M, RT
16:23:22	**Imaging Exam Ended**	XR Spine Lumbar 4 or More Views	Dunham, Carrie An M, RT
16:35:58	**XR Knee Right 4 or More Views Resulted**	Collected: **12/2/2022 16:35** Last updated: **12/2/2022 16:38** Status: **Final result**	Interface, Radiology Results In
16:35:58	**Order Performed**	XR Knee Right 4 or More Views - ID: **60815314**	
16:38:06	**Imaging Final Result**	XR Knee Right 4 or More Views	Interface, Radiology Results In
16:38:06	**Xray Final Result**	(Final result) XR KNEE RIGHT 4 OR MORE VIEWS	Interface, Radiology Results In
16:39:10	**XR Spine Lumbar 4 or More Views Resulted**	Collected: **12/2/2022 16:39** Last updated: **12/2/2022 16:41** Status: **Final result**	Interface, Radiology Results In
16:39:10	**Order Performed**	XR Spine Lumbar 4 or More Views - ID: **60815320**	
16:40	**Early Detection of Sepsis**	**Other flowsheet entries** Sepsis Score: 1.07	Background User, Cogito

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

16:41:18	**Imaging Final Result**	XR Spine Lumbar 4 or More Views	Interface, Radiology Results In
16:41:19	**Xray Final Result**	(Final result) XR SPINE LUMBAR 4 OR MORE VIEWS	Interface, Radiology Results In
16:54:59	**Assign Attending**	Abbott, Robert Emmons, MD assigned as Attending	King, Kimberly Janean, NP
16:54:59	**Assign Physician**		King, Kimberly Janean, NP
16:58:03	**ED Midlevel Note Shared**	ED Prov Note filed by King, Kimberly Janean, NP	King, Kimberly Janean, NP
16:58:27	**Discharge Disposition Selected**	ED Disposition set to Discharge	King, Kimberly Janean, NP
16:58:27	**Disposition Selected**		King, Kimberly Janean, NP
16:59:39	**Discharge Orders Placed**	Medications - acetaminophen (TYLENOL) 500 mg tablet; methocarbamoL (ROBAXIN) 500 mg tablet; naproxen (NAPROSYN) 500 mg tablet	King, Kimberly Janean, NP
17:00:17	**ED Midlevel Note Signed**	ED Prov Note filed by King, Kimberly Janean, NP	King, Kimberly Janean, NP
17:00:17	**ED Provider Notes**	Note filed at this time	King, Kimberly Janean, NP; Cosigned by Abbott, Robert Emmons, MD
17:00:22	**AVS Printed**		King, Kimberly Janean, NP
17:00:22	**AVS Printed**	Stop the Spread of Germs ED After Visit Summary	King, Kimberly Janean, NP
17:00:25	**Patient Ready to Go**		King, Kimberly Janean, NP
17:01	**Early Detection of Sepsis**	**Other flowsheet entries** Sepsis Score: 1.07	Background User, Cogito
17:11	**Departure Condition**	**Departure Condition** Departure Condition: Stable Mobility at Departure: Ambulatory Patient Teaching: Discharge instructions/follow-up care reviewed; Patient verbalized understanding; Prescriptions discussed Extended patient education (greater than 15 minutes) given to patient/guardian?: Not required Mode of Transport: In private vehicle Accompanied By:: Family/significant other	Hopper, Teresa L., RN
17:12	**Patient discharged**		Hopper, Teresa L., RN
17:12	**Patient Ready for Transport**	**Nursing Patient Disposition** Admitted or Discharged: Ready to be discharged.	Hopper, Teresa L., RN
17:12	**LACE+ Score**	**Other flowsheet entries** LACE+ Score: 52	Hopper, Teresa L., RN
17:12:21	**Summary with Delay**		Hopper, Teresa L., RN
17:12:23	**Charting Complete**		Hopper, Teresa L., RN
17:12:23	**Charting Complete**		King, Kimberly Janean, NP

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

ED Care Timeline (continued)

Clinical Notes

Discharge Instructions

King, Kimberly Janean, NP at 12/2/2022 1700

Author: King, Kimberly Janean, NP | Service: — | Author Type: Nurse Practitioner
Filed: 12/2/2022 5:00 PM | Date of Service: 12/2/2022 5:00 PM | Status: Written
Editor: King, Kimberly Janean, NP (Nurse Practitioner)

Take the medications as prescribed. Follow up with your doctor or the doctor provided for any new or worsening concerns.[KK.1T]

Electronically signed by King, Kimberly Janean, NP at 12/2/2022 5:00 PM

Attribution Key

KK.1 - King, Kimberly Janean, NP on 12/2/2022 4:58 PM
T - Template

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Imaging

Imaging

XR Knee Right 4 or More Views (Final result)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1550** Status: **Completed**
This order may be acted on in another encounter.
Ordering user: King, Kimberly Janean, NP 12/02/22 1550
Authorized by: King, Kimberly Janean, NP
Frequency: ED Once 12/02/22 1550 - 1 occurrence
Quantity: 1
Indications comment: pain s/p mvc

Ordering provider: King, Kimberly Janean, NP
Ordering mode: Standard
Class: Hospital Performed
Lab status: Final result
Instance released by: King, Kimberly Janean, NP (auto-released) 12/2/2022 3:50 PM

End Exam Questions

	Answer	Comment
Were any student technologist involved in this procedure?		
Please insert the name of the student in this field.		

XR Knee Right 4 or More Views Resulted: 12/02/22 1635, Result status: Final result

Ordering provider: King, Kimberly Janean, NP 12/02/22 1550
Resulted by: Hargan, Paul W., MD
Performed: 12/02/22 1557 - 12/02/22 1623
Resulting lab: POWERSCRIBE
Narrative:
EXAMINATION:
XR KNEE RIGHT 4 OR MORE VIEWS

HISTORY:
Right knee pain after MVA

ORDER DATE:
12/2/2022 4:00 PM

Order status: Completed
Filed by: Interface, Radiology Results In 12/02/22 1638
Accession number: 60815314

Impression:
FINDINGS/IMPRESSION:

There is no sign of any acute osseous or articular abnormality. There are no specific soft tissue abnormalities.

Electronically signed by: Paul Hargan, M.D.

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
338 - POWERSCRIBE	POWERSCRIBE	Unknown	Unknown	04/18/17 1228 - Present

XR Knee Right 4 or More Views Resulted: 12/02/22 1635, Result status: In process

Ordering provider: King, Kimberly Janean, NP 12/02/22 1550
Resulted by: Hargan, Paul W., MD
Performed: 12/02/22 1557 - 12/02/22 1623
Resulting lab: RAD_PACS_CH

Order status: Completed
Filed by: Hargan, Paul W., MD 12/02/22 1636
Accession number: 60815314

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Imaging (continued)

XR Knee Right 4 or More Views Resulted: 12/02/22 1559, Result status: In process

Ordering provider: King, Kimberly Janean, NP 12/02/22 1550
Resulted by: Hargan, Paul W., MD
Performed: 12/02/22 1557 - 12/02/22 1623
Resulting lab: RAD_PACS_CH

Order status: Completed
Filed by: Dunham, Carrie An M, RT 12/02/22 1559
Accession number: 60815314

Signed

Electronically signed by Hargan, Paul W., MD on 12/2/22 at 1635 CST

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Imaging (continued)

XR Spine Lumbar 4 or More Views (Final result)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1550** Status: **Completed**
This order may be acted on in another encounter.
Ordering user: King, Kimberly Janean, NP 12/02/22 1550
Authorized by: King, Kimberly Janean, NP
Frequency: ED Once 12/02/22 1551 - 1 occurrence
Quantity: 1
Indications comment: s/p MVC

Ordering provider: King, Kimberly Janean, NP
Ordering mode: Standard
Class: Hospital Performed
Lab status: Final result
Instance released by: King, Kimberly Janean, NP (auto-released) 12/2/2022 3:50 PM

End Exam Questions

	Answer	Comment
Were any student technologist involved in this procedure?		
Please insert the name of the student in this field.		

XR Spine Lumbar 4 or More Views Resulted: 12/02/22 1639, Result status: Final result

Ordering provider: King, Kimberly Janean, NP 12/02/22 1550
Resulted by: Hargan, Paul W., MD
Performed: 12/02/22 1557 - 12/02/22 1623
Resulting lab: POWERSCRIBE

Order status: Completed
Filed by: Interface, Radiology Results In 12/02/22 1641
Accession number: 60815320

Narrative:
EXAMINATION: XR SPINE LUMBAR 4 OR MORE VIEWS

L-SPINE : 5 views

DATE OF EXAM: 12/2/2022 4:38 PM

HISTORY: s/p MVC, back pain

FINDINGS:

The lumbar vertebral bodies and the disc spaces are normal in width. In the lumbar vertebra, there is no evidence of compression deformities or spondylolisthesis.

Impression:

UNREMARKABLE LUMBAR SPINE STUDY

Electronically signed by: Paul Hargan, M.D.

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
338 - POWERSCRIBE	POWERSCRIBE	Unknown	Unknown	04/18/17 1228 - Present

XR Spine Lumbar 4 or More Views Resulted: 12/02/22 1638, Result status: In process

Ordering provider: King, Kimberly Janean, NP 12/02/22 1550
Resulted by: Hargan, Paul W., MD
Performed: 12/02/22 1557 - 12/02/22 1623
Resulting lab: RAD_PACS_CH

Order status: Completed
Filed by: Hargan, Paul W., MD 12/02/22 1639
Accession number: 60815320

XR Spine Lumbar 4 or More Views Resulted: 12/02/22 1559, Result status: In process

Ordering provider: King, Kimberly Janean, NP 12/02/22 1550
Resulted by: Hargan, Paul W., MD

Order status: Completed
Filed by: Dunham, Carrie An M, RT 12/02/22 1559

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Imaging (continued)

Performed: 12/02/22 1557 - 12/02/22 1623
Accession number: 60815320
Resulting lab: RAD_PACS_CH

Signed

Electronically signed by Hargan, Paul W., MD on 12/2/22 at 1639 CST

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Case 9882993

Surgery Information

General Information

Date: 12/2/2022	Time:	Status: Posted
Location: CH INVASIVE	Room:	Service:
Patient class:	Case classification:	

Diagnosis Information

No post-op diagnosis codes associated with the log.

Clinical Documentation

Case Tracking Events

Event	Time In
In Facility	1527

Event Tracking

No event timings documented

Patient Preparation

None

Counts by Panel

Panel 1

Type	Which?	Correct ?	X-Ray?	MD Notified?	Counted By	Verified By
Soft Goods	Initial					
Needles/Sharps	Initial					
Instruments	Initial					
Soft Goods	Final					
Needles/Sharps	Final					
Instruments	Final					

12/02/2022 - ED in Christian Hospital Northwest Emergency Department
OpTime Log Detail

General Information

Date: 12/2/2022	Time:	Status: Posted
Location: CH INVASIVE	Room:	Service:
Patient class:	Case classification:	

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Medication Administrations

acetaminophen (TYLENOL) tablet 650 mg [642836793]

Ordering Provider: King, Kimberly Janean, NP
Ordered On: 12/02/22 1550
Ordered Dose (Remaining/Total): 650 mg (0/1)
Frequency: Once
Status: Completed (Past End Date/Time)
Starts/Ends: 12/02/22 1551 - 12/02/22 1558
Route: oral
Ordered Rate/Order Duration: — / —

Timestamps	Action	Dose	Route	Other Information
Performed 12/02/22 1558 Documented: 12/02/22 1559	Given	650 mg	oral	Performed by: Hopper, Teresa L., RN Scanned Package: 69618-010-01, 71399-8024-2

ibuprofen (ADVIL,MOTRIN) tablet 800 mg [642836792]

Ordering Provider: King, Kimberly Janean, NP
Ordered On: 12/02/22 1550
Ordered Dose (Remaining/Total): 800 mg (0/1)
Frequency: Once
Status: Completed (Past End Date/Time)
Starts/Ends: 12/02/22 1551 - 12/02/22 1558
Route: oral
Ordered Rate/Order Duration: — / —

Timestamps	Action	Dose	Route	Other Information
Performed 12/02/22 1558 Documented: 12/02/22 1559	Given	800 mg	oral	Performed by: Hopper, Teresa L., RN Scanned Package: 0904-5855-61

Other Orders

Medications

ibuprofen (ADVIL,MOTRIN) tablet 800 mg (Completed)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1550**
Status: **Completed**
Ordering user: King, Kimberly Janean, NP 12/02/22 1550
Ordering provider: King, Kimberly Janean, NP
Authorized by: King, Kimberly Janean, NP
Ordering mode: Standard
Frequency: Routine Once 12/02/22 1551 - 1 occurrence
Class: Normal
Acknowledged: Hopper, Teresa L., RN 12/02/22 1556 for Placing Order
Package: 0904-5855-61

acetaminophen (TYLENOL) tablet 650 mg (Completed)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1550**
Status: **Completed**
Ordering user: King, Kimberly Janean, NP 12/02/22 1550
Ordering provider: King, Kimberly Janean, NP
Authorized by: King, Kimberly Janean, NP
Ordering mode: Standard
Frequency: Routine Once 12/02/22 1551 - 1 occurrence
Class: Normal
Acknowledged: Hopper, Teresa L., RN 12/02/22 1556 for Placing Order
Package: 69618-010-01, 71399-8024-2

acetaminophen (TYLENOL) 500 mg tablet (Active)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1659**
Status: **Active**
Ordering user: King, Kimberly Janean, NP 12/02/22 1659
Ordering provider: King, Kimberly Janean, NP
Authorized by: King, Kimberly Janean, NP
Ordering mode: Standard
PRN reasons: headaches pain
Frequency: Routine Q6H PRN 12/02/22 - Until Discontinued
Class: Normal

Questionnaire

Question	Answer
Collaborating provider? (advanced practice providers only)	ABBOTT, ROBERT EMMONS

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Other Orders (continued)

methocarbamoL (ROBAXIN) 500 mg tablet (Active)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1659** Status: **Active**
Ordering user: King, Kimberly Janean, NP 12/02/22 1659
Ordering provider: King, Kimberly Janean, NP
Authorized by: King, Kimberly Janean, NP
Ordering mode: Standard
Frequency: Routine BID 12/02/22 - Until Discontinued
Class: Normal

Questionnaire

Question	Answer
Collaborating provider? (advanced practice providers only)	ABBOTT, ROBERT EMMONS

naproxen (NAPROSYN) 500 mg tablet (Active)

Electronically signed by: **King, Kimberly Janean, NP on 12/02/22 1659** Status: **Active**
Ordering user: King, Kimberly Janean, NP 12/02/22 1659
Ordering provider: King, Kimberly Janean, NP
Authorized by: King, Kimberly Janean, NP
Ordering mode: Standard
Frequency: Routine BID with meals (bkfst, dinner) 12/02/22 - Until Discontinued
Class: Normal

Questionnaire

Question	Answer
Collaborating provider? (advanced practice providers only)	ABBOTT, ROBERT EMMONS

Flowsheets

Abuse Assessment

Row Name	12/02/22 1500

Abuse Assessment: Conduct in private to create environment of safety and confidentiality.

Have you ever been in or are you currently in a harmful physical or emotional relationship or is someone making you feel afraid or unsafe?	Denies -MB at 12/02/22 1500
Do you have someplace safe to go at discharge?	Yes -MB at 12/02/22 1500
Who answered abuse screen question?	Patient -MB at 12/02/22 1500
Who was in the room when abuse screen was answered?	Patient -MB at 12/02/22 1500

Acuity/Vertical

Row Name	12/02/22 1500

Acuity/Vertical

Patient Acuity	Less Urgent -MB at 12/02/22 1500
Vertical	Vertical -MB at 12/02/22 1500

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

Anthropometrics

Row Name	12/02/22 1448

Anthropometrics

Weight Change	-9.48 kg (-20.91 lbs) -KD at 12/02/22 1450

C-SSRS NEW

Row Name	12/02/22 1500

Encounter Related Suicide Attempt/Behavior: Whenever possible, conduct screening in private to create a confidential environment.

Is this encounter related to a suicidal attempt/behavior ?	No -MB at 12/02/22 1500
Information obtained from:	Patient -MB at 12/02/22 1500

Columbia Suicide Severity Rating Scale (Recent Screener)

1. In the past month, have you wished you were dead or that you could go to sleep and not wake up?	No -MB at 12/02/22 1500
2. In the past month, have you actually had any thoughts of killing yourself?	No -MB at 12/02/22 1500
6. Have you ever done anything, started to do anything, or prepared to do anything to end your life?	No -MB at 12/02/22 1500

Custom Formula Data

Row Name	12/02/22 1500	12/02/22 1448

OTHER

BMI (Calculated)	—	21.9 -KD at 12/02/22 1450
IBW in lbs (Bariatric)	—	190 -KD at 12/02/22 1450
Weight Change Since Last Visit	—	77.09 kg -KD at 12/02/22 1450
IBW in kg (Bariatric)	—	86.18 -KD at 12/02/22 1450
Initial Rate	—	12 -KD at 12/02/22 1450
Initial Rate:	—	7.5 -KD at 12/02/22 1450
Initial Rate	—	10 -KD at 12/02/22 1450
IBW/kg (Calculated) Male	—	82.2 kg -KD at 12/02/22 1450
IBW/kg (Calculated)	—	86.2 kg -KD at 12/02/22 1450
:	—	12 -KD at 12/02/22 1450

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

:	—	10 -KD at 12/02/22 1450
BSA (Calculated - sq m)	—	2 sq meters -KD at 12/02/22 1450
Admission Weight	—	77.111 -KD at 12/02/22 1450
Measurements		
Total Weight Change Percent	—	2222 Percent -KD at 12/02/22 1450
Weight Change Since Preop	—	77.09 kgs -KD at 12/02/22 1450
Initial Excess Weight	—	-86.18 kgs -KD at 12/02/22 1450
Percent of IBW	—	3156.18 Percent -KD at 12/02/22 1450
EBW (kg)	—	2717.56 kg -KD at 12/02/22 1450
EBW (lbs)	—	2708.13 lbs -KD at 12/02/22 1450
Anthropometrics		
Admission Weight	—	77.1 kg -KD at 12/02/22 1450
Vital Signs		
BMI (Calculated)	—	21.8 -KD at 12/02/22 1450
Height and Weight		
Weight in (lb) to have BMI = 25	—	194.3 -KD at 12/02/22 1450
Relevant Labs and Vitals		
Temp (in Celsius)	—	36.7 -KD at 12/02/22 1450
Adult IBW/VT Calculations		
IBW/kg (Calculated)	—	82.2 -KD at 12/02/22 1450
Low Range Vt 6mL/kg	—	493.2 mL/kg -KD at 12/02/22 1450
Adult Moderate Range Vt 8mL/kg	—	657.6 mL/kg -KD at 12/02/22 1450
Adult High Range Vt 10mL/kg	—	822 mL/kg -KD at 12/02/22 1450
Columbia Suicide Severity Rating Scale (Recent Screener)		
Suicide Risk Level	No risk level -MB at 12/02/22 1500	—

Departure Condition

Row Name	12/02/22 1711
Departure Condition	
Departure Condition	Stable -TH at 12/02/22 1712
Mobility at Departure	Ambulatory -TH at 12/02/22 1712
Patient Teaching	Discharge instructions/follow-up care reviewed;Patient verbalized understanding;Prescriptions discussed -TH at 12/02/22 1712

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

Extended patient education (greater than 15 minutes) given to patient/guardian?	Not required -TH at 12/02/22 1712
Mode of Transport	In private vehicle - TH at 12/02/22 1712
Accompanied By:	Family/significant other -TH at 12/02/22 1712

Early Detection of Sepsis

Row Name	12/02/22 1701	12/02/22 1640	12/02/22 1620	12/02/22 1600	12/02/22 1540
OTHER					
Sepsis Score	1.07 -CB at 12/02/22 1705	1.07 -CB at 12/02/22 1644	1.07 -CB at 12/02/22 1624	1.07 -CB at 12/02/22 1604	1.13 -CB at 12/02/22 1544

Elopement Precautions

Row Name	12/02/22 1500
Elopement Risk Assessment	
Risk/Reason for Elopement	No risk -MB at 12/02/22 1500

Fall Risk Assessment (MEDFRAT)

Row Name	12/02/22 1500
Fall Risk Assessment Tool - MEDFRAT	
History of falling in last 3 months, including since admission	No -MB at 12/02/22 1501
Prior Fall Event (Autopopulated from EMR)	None found -MB at 12/02/22 1501
Confusion or disorientation	No -MB at 12/02/22 1501
Intoxicated or sedated	No -MB at 12/02/22 1501
Impaired gait	No -MB at 12/02/22 1501
Mobility assist device used	No -MB at 12/02/22 1501
Altered elimination	No -MB at 12/02/22 1501
Fall risk score: (1-2 low risk), (3-4 moderate risk), (5 or more high risk)	0 -MB at 12/02/22 1501

HIV Screening

Row Name	12/02/22 1502
HIV Screening	
HIV Screen Offered	Patient refused HIV screening -MB at

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

	12/02/22 1502
Reason screening refused	No reason stated - MB at 12/02/22 1502

Is patient diabetic

Row Name	12/02/22 1459
Is the patient diabetic	
Is patient diabetic	No -MB at 12/02/22 1459

LACE+ Score

Row Name	12/02/22 1712
OTHER	
LACE+ Score	52 -TH at 12/02/22 1712

Opioid Screening Tool

Row Name	12/02/22 1502
Opioid Screening Tool - In the Past 3 Months	
Have you regularly used prescription pain medication like Percocet or Norco for non-medical reasons?	No -MB at 12/02/22 1502
Have you used illegal opioids such as heroin or fentanyl	No -MB at 12/02/22 1502

Pain Assessment

Row Name	12/02/22 1500
Pain Assessment	0-10 -MB at 12/02/22 1500
Pain Score	8 -MB at 12/02/22 1500

Pain Assessment

Row Name	12/02/22 1500
Pain Assessment Timer	
Restart Pain Assessment Timer	Yes -MB at 12/02/22 1500

Patient Ready for Transport

Row Name	12/02/22 1712
Nursing Patient Disposition	
Admitted or Discharged	Ready to be discharged. -TH at

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

12/02/22 1712

Primary Assessment

Row Name	12/02/22 1459
Airway	
Airway	Patent -MB at 12/02/22 1459
Breathing	
Breathing	Adequate -MB at 12/02/22 1459
Circulation	
Circulation	Adequate -MB at 12/02/22 1459
Disability	
Disability	Alert -MB at 12/02/22 1459

Screen for infection

Row Name	12/02/22 1459
Infection Screen	
Does the patient have an infection, or do you suspect they may have an infection	No -MB at 12/02/22 1459
Does the patient have risk factors for infection	No -MB at 12/02/22 1459

Secondary Assessment Complete

Row Name	12/02/22 1502
Secondary Assessment	
Secondary Complete	Secondary Complete -MB at 12/02/22 1502

Stroke Screening Tool

Row Name	12/02/22 1459
FAST	
Facial Droop	Both sides of the face move equally - MB at 12/02/22 1500
Arm Drift	Both arms have the same response: drift or do not move at all. -MB at 12/02/22 1500
Speech	Says correct words with no slurring. -MB at 12/02/22 1500
Assess for Posterior Circulation Stroke	
Assess Horizontal Gaze	Both eyes track evenly to the outer limit of the normal

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

	ocular range. -MB at 12/02/22 1500
Vison	No double vision or sudden vision changes in one or both eyes. -MB at 12/02/22 1500
Loss of balance, stumbling or gait change	No sudden loss of balance, stumbling or changes in walking with no known cause. -MB at 12/02/22 1500
Perform Finger to Nose test	Patient performs test movements without abnormal findings - bilateral tremors or bilateral coordination deficits are marked as normal -MB at 12/02/22 1500

Tetanus/Immunization Status

Row Name	12/02/22 1501
Tetanus Up to Date	
Is the patient's tetanus up to date?	Yes -MB at 12/02/22 1502
Immunizations Up to Date	
Are the patient's immunizations up to date?	Yes -MB at 12/02/22 1502

Travel and Exposure Screening

Row Name	12/02/22 1459
Travel Screening	
Have you traveled outside the U.S. in the last 6 months?	No -MB at 12/02/22 1459
Exposure Screening	
Have you been exposed to anyone who is sick in the last 30 days?	No -MB at 12/02/22 1459
Infectious Disease Screening	
Are you having any of the following:	None -MB at 12/02/22 1459

Vital Signs

Row Name	12/02/22 1448
Vital Signs	
Temp	36.7 °C (98 °F) -KD

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (continued)

	at 12/02/22 1450
Temp src	Tympanic -KD at 12/02/22 1450
Pulse	79 -KD at 12/02/22 1450
Heart Rate Source	Monitor -KD at 12/02/22 1450
Resp	18 -KD at 12/02/22 1450
Resp Source	Visual -KD at 12/02/22 1450
BP	112/68 -KD at 12/02/22 1450
BP Location	Right arm -KD at 12/02/22 1450
BP Method	Automatic -KD at 12/02/22 1450
SpO2	97 % -KD at 12/02/22 1450
Height and Weight	
Height	188 cm (6' 2") -KD at 12/02/22 1450
Height Method	Stated -KD at 12/02/22 1450
Weight	77.1 kg (170 lb) -KD at 12/02/22 1450
Weight Method	Stated -KD at 12/02/22 1450
Vitals Timer	
Restart Vitals Timer	Yes -KD at 12/02/22 1450

User Key (r) = Recorded By, (t) = Taken By, (c) = Cosigned By

Initials	Name	Effective Dates	Provider Type	Discipline
CB	Background User, Cogito	—	—	—
TH	Hopper, Teresa L., RN	04/30/21 - 03/05/23	Registered Nurse	Nurse
MB	Brickman, Michael J., RN	09/24/22 -	Registered Nurse	Nurse
KD	Donaldson, Kaviona	06/01/22 - 01/09/23	Registered Nurse	Nurse

The Asthma Action Plan has not been signed, or it has been modified since it was last signed. Please have a provider or nurse review and sign in order to print this document.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department
Asthma Action Plan

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

After Visit Summary

Stop the Spread of Germs (below)

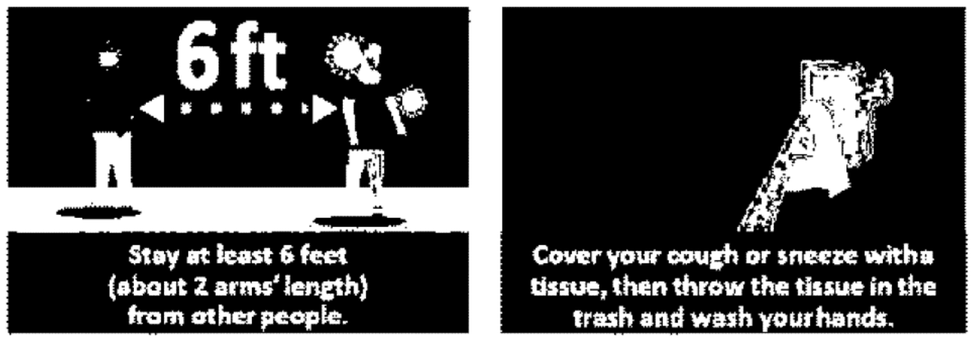
[redacted] #[redacted] (Acct: [redacted]) ([redacted] y.o. M) (Adm: 12/02/22) PCP: NO, P (999-999-9999)

PED24 (In Process)

Stop the Spread of Germs

Stop the Spread of Ger[illegible]

Help prevent the spread of respiratory diseases like COVID-19.



cdc.gov/coronavirus

ED After Visit Summary (below)

AFTER VISIT SUMMARY

BJC HealthCare

[redacted] DoB: [redacted]

12/2/2022 Christian Hospital Northwest Emergency Department 314-953-6994

Instructions

Take the medications as prescribed. Follow up with your doctor or the doctor provided for any new or worsening concerns.

Your medication list has changed.

START taking:
methocarbamol (ROBAXIN)

naproxen (NAPROSYN)

CHANGE how you take
acetaminophen (TYLENOL) - medication strength, how much to take, reasons to take this

Review your updated medication list below.

Discharge Education

1. Contusion in Adults (AfterCare(R) Instructions(ER/ED)) (English)
2. Muscle Strain (AfterCare(R) Instructions(ER/ED)) (English)
3. Motor Vehicle Accident (AfterCare(R) Instructions(ER/ED)) (English)

[redacted]

acetaminophen • methocarbamol • naproxen

Address: [redacted]
Phone: [redacted]

[redacted]

Way: As needed, If symptoms worsen

[redacted]

Way: As needed, If symptoms worsen, To establish primary care
Specialty: Interventional Cardiology, Internal Medicine
Contact: 637 DUNN RD STE 170
Hazelwood MO 63042
314 838 5702

Today's Visit

You were seen by Robert Emmons Abbott, MD and Kimberly Janean King, NP

Reason for Visit
Motor Vehicle Crash

Diagnoses
- Motor vehicle accident, initial encounter
- Strain of trapezius muscle, unspecified laterality, initial encounter
- Strain of lumbar region, initial encounter
- Contusion of right knee, initial encounter

Imaging Tests
XR Knee Right 4 or More Views
XR Spine Lumbar 4 or More Views

Medications Given
acetaminophen (TYLENOL) tablet 650 mg Last given at 3:58 PM
ibuprofen (ADVIL,MOTRIN) tablet 800 mg Last given at 3:58 PM

MyChart

View your After Visit Summary and more online at https://www.mypatientchart.org/MyChart/.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

After Visit Summary (continued)

What's Next

You currently have no upcoming appointments scheduled.

Your Medication List

TAKE these medications

acetaminophen 500 mg tablet
TYLENOL

Take 1 tablet (500 mg total) by mouth every 6 (six) hours as needed for headaches or pain

- **medication strength**
- **how much to take**
- **reasons to take this**

START

methocarbamol 500 mg tablet
ROBAXIN

Take 1 tablet (500 mg total) by mouth 2 (two) times a day

START

naproxen 500 mg tablet
NAPROSYN

Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals

ASK your doctor about these medications

ASK

bisacodyl EC 5 mg EC tablet
DULCOLAX EC

Take 2 tablets (10 mg total) by mouth daily as needed for constipation (If no results 24 hours after polyethylene glycol (MIRALAX). May give bisacodyl supp if not tolerating PO))

ASK

famotidine 20 mg tablet
PEPCID

Take 1 tablet (20 mg total) by mouth 2 (two) times a day

ASK

montelukast 10 mg tablet
SINGULAIR

Take 1 tablet (10 mg total) by mouth nightly

ASK

ondansetron 4 mg tablet
ZOFRAN

Take 1 tablet (4 mg total) by mouth every 8 (eight) hours as needed for nausea or vomiting

ASK

polyethylene glycol 17 gram packet
MIRALAX

Take 1 packet (17 g total) by mouth daily as needed for constipation

Physician Referral Line

If you currently do not have a primary care physician, please call the physician referral line at: **314-747-WELL or 1-877-747-WELL.**

The Emergency Department is for the treatment of medical/surgical emergencies. It is very important that you be seen by your primary care physician or home clinic for on-going care within the time frame recommended by your ED provider.

The emergency physician may have provided a preliminary interpretation of your x-rays and/or EKG. If so, a specialist will perform a final interpretation of these tests. If a change in your diagnosis or treatment is needed, we will contact you. It is critical that we have a current phone number for you.

If you experience any thoughts of harming yourself, contact the **National Suicide Prevention Lifeline** at **988**.

Resources available for youth experiencing any thoughts of self-harm include:
Behavioral Health Response Missouri Crisis Hotlines: **314-469-6644** (local), **1-800-811-4760** (toll free)
Youth in Crisis at Behavioral Health Response: **1-844-985-8282** or Text: **BHEARD** to **31658** (message and data rates apply)

Attached Information — Contusion in Adults (AfterCare(R) Instructions.FR/FD): (English)

Contusion in Adults

WHAT YOU NEED TO KNOW:

A contusion is a bruise that appears on your skin after an injury. A bruise happens when small blood vessels tear but skin does not. When blood vessels tear, blood leaks into nearby tissue, such as soft tissue or muscle.

DISCHARGE INSTRUCTIONS:

Return to the emergency department if:

- You have new trouble moving the injured area.
- You have tingling or numbness in or near the injured area.
- Your hand or foot below the bruise gets cold or turns pale.

Contact your healthcare provider if:

- You find a new lump in the injured area.
- Your symptoms do not improve with treatment after 4 to 5 days.
- You have questions or concerns about your condition or care.

Medicines: You **may** need any of the following:

- **NSAIDs** help decrease swelling and pain or fever. This medicine is available with or without a doctor's order. NSAIDs can cause stomach bleeding or kidney problems in certain people. If you take blood thinner medicine, **always** ask your healthcare provider if NSAIDs are safe for you. Always read the medicine label and follow directions.
- **Prescription pain medicine** may be given. Do not wait until the pain is severe before you take your medicine.
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell him of her if you are allergic to any medicine. Keep a list of the medicines, vitamins, and herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Follow up with your healthcare provider as directed: You may need to return within a week to check your injury again. Write down your questions so you remember to ask them during your visits.

Help a contusion heal:

- **Rest the injured area** or use it less than usual. If you bruised your leg or foot, you may need crutches or a cane to help you walk. This will help you keep weight off your injured body part.
- **Apply ice** to decrease swelling and pain. Ice may also help prevent tissue damage. Use an ice pack, or put crushed ice in a plastic bag. Cover it with a towel and place it on your bruise for 15 to 20 minutes every hour or as directed.

- **Use compression** to support the area and decrease swelling. Wrap an elastic bandage around the area over the bruised muscle. Make sure the bandage is not too tight. You should be able to fit 1 finger between the bandage and your skin.

- **Elevate (raise) your injured body part** above the level of your heart to help decrease pain and swelling. Use pillows, blankets, or rolled towels to elevate the area as often as you can.

- **Do not drink alcohol** as directed. Alcohol may slow healing.

- **Do not stretch injured muscles** right after your injury. Ask your healthcare provider when and how you may safely stretch after your injury. Gentle stretches can help increase your flexibility.

- **Do not massage the area or put heating pads** on the bruise right after your injury. Heat and massage may slow healing. Your healthcare provider may tell you to apply heat after several days. At that time, heat will start to help the injury heal.

Prevent another contusion:

- Stretch and warm up before you play sports or exercise.

- Wear protective gear when you play sports. Examples are shin guards and padding.

- If you begin a new physical activity, start slowly to give your body a chance to adjust.

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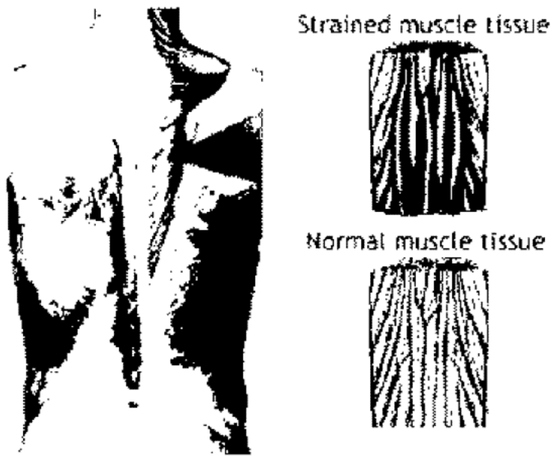
The above information is an educational aid only. It is not intended as medical advice for individual conditions or treatments. Talk to your doctor, nurse or pharmacist before following any medical regimen to see if it is safe and effective for you.

Attached Information — Muscle Strain (AfterCare(R) Instructions.FR/FD): (English)

Muscle Strain

WHAT YOU NEED TO KNOW:

A muscle strain is a twist, pull, or tear of a muscle or tendon. A tendon is a strong elastic tissue that connects a muscle to a bone. Signs of a strained muscle include bruising and swelling over the area, pain with movement, and loss of strength.



DISCHARGE INSTRUCTIONS:

Return to the emergency department if:

- You suddenly cannot feel or move your injured muscle.

Contact your healthcare provider if:

- Your pain and swelling worsen or do not go away.
- You have questions or concerns about your condition or care.

Medicines:

- **NSAIDs** help decrease swelling and pain or fever. This medicine is available with or without a doctor's order. NSAIDs can cause stomach bleeding or kidney problems in certain people. If you take blood thinner medicine, **always** ask your healthcare provider if NSAIDs are safe for you. Always read the medicine label and follow directions.
- **Muscle relaxers** help decrease pain and muscle spasms.
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell him of her if you are allergic to any medicine. Keep a list of the medicines, vitamins, and

herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Follow up with your healthcare provider as directed: Your healthcare provider may suggest that you have a follow up visit before you go back to your usual activity. Write down your questions so you remember to ask them during your visits.

Self-care:

- **3 to 7 days after the injury:** Use Rest, Ice, Compression, and Elevation (RICE) to help stop bruising and decrease pain and swelling.
    - **Rest:** Rest your muscle to allow your injury to heal. When the pain decreases, begin normal, slow movements. For mild and moderate muscle strains, you should rest your muscles for about 2 days. However, if you have a severe muscle strain, you should rest for 10 to 14 days. You may need to use crutches to walk if your muscle strain is in your legs or lower body.
    - **Ice:** Put an ice pack on the injured area. Put a towel between the ice pack and your skin. Do **not** put the ice pack directly on your skin. You can use a package of frozen peas instead of an ice pack.
    - **Compression:** You may need to wrap an elastic bandage around the area to decrease swelling. It should be tight enough for you to feel support. Do **not** wrap it too tightly.
    - **Elevation:** Keep the injured muscle raised above your heart if possible. For example if you have a strain of your lower leg muscle, lie down and prop your leg up on pillows. This helps decrease pain and swelling.
- **3 to 21 days after the injury:** Start to slowly and regularly exercise your muscle. This will help it heal. If you feel pain, decrease how hard you are exercising.
- **1 to 6 weeks after the injury:** Stretch the injured muscle. Hold the stretch for about 30 seconds. Do this 4 times a day. You may stretch the muscle until you feel a slight pull. Stop stretching if you feel pain.
- **2 weeks to 6 months after the injury:** The goal of this phase is to return to the activity you were doing before the injury happened, without hurting the muscle again.
- **3 weeks to 6 months after the injury:** Keep stretching and strengthening your muscles to avoid injury. Slowly increase the time and distance that you exercise. You may have signs and symptoms of muscle strain 6 months after the injury, even if you do things to help it heal. In this case, you may need surgery on the muscle.

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Attached Information — Motor Vehicle Accident (AfterCare/R) Instructions.FR/FD): (English

Motor Vehicle Accident

WHAT YOU NEED TO KNOW:

A motor vehicle accident (MVA) can cause injury from the impact or from being thrown around inside the car. You may have a bruise on your abdomen, chest, or neck from the seatbelt. You may also have pain in your face, neck, or back. You may have pain in your knee, hip, or thigh if your body hits the dash or the steering wheel. Muscle pain is commonly worse 1 to 2 days after an MVA.

DISCHARGE INSTRUCTIONS:

Call 911 if:

- You have new or worsening chest pain or shortness of breath.

Return to the emergency department if:

- You have new or worsening pain in your abdomen.
- You have nausea and vomiting that does not get better.
- You have a severe headache.
- You have weakness, tingling, or numbness in your arms or legs.
- You have new or worsening pain that makes it hard for you to move.

Contact your healthcare provider if:

- You have pain that develops 2 to 3 days after the MVA.
- You have questions or concerns about your condition or care.

Medicines:

- **Pain medicine:** You may be given medicine to take away or decrease pain. Do not wait until the pain is severe before you take your medicine.
- **NSAIDs** , such as ibuprofen, help decrease swelling, pain, and fever. This medicine is available with or without a doctor's order. NSAIDs can cause stomach bleeding or kidney problems in certain people. If you take blood thinner medicine, **always** ask if NSAIDs are safe for you. Always read the medicine label and follow directions. **Do not give these medicines to children under 6 months of age without direction from your child's healthcare provider.**
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell him of her if you are allergic to any medicine. Keep a list of the medicines, vitamins, and herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Follow up with your healthcare provider as directed: Write down your questions so you remember to ask them during your visits.

Safety tips:

- **Always wear your seatbelt.** This will help reduce serious injury from an MVA.
- **Use child safety seats.** Your child needs to ride in a child safety seat made for his age, height, and weight. Ask your healthcare provider for more information about child safety seats.
- **Decrease speed.** Drive the speed limit to reduce your risk for an MVA.
- **Do not drive if you are tired.** You will react more slowly when you are tired. The slowed reaction time will increase your risk for an MVA.
- **Do not talk or text on your cell phone while you drive.** You cannot respond fast enough in an emergency if you are distracted by texts or conversations.
- **Do not drink and drive.** Use a designated driver. Call a taxi or get a ride home with someone if you have been drinking. Do not let your friends drive if they have been drinking alcohol.
- **Do not use illegal drugs and drive.** You may be more tired or take risks that you normally would not take. Do not drive after you take prescription medicines that make you sleepy.

Self-care:

- **Use ice and heat.** Ice helps decrease swelling and pain. Ice may also help prevent tissue damage. Use an ice pack, or put crushed ice in a plastic bag. Cover it with a towel and apply to your injured area for 15 to 20 minutes every hour, or as directed. After 2 days, use a heating pad on your injured area. Use heat as directed.
- **Gently stretch.** Use gentle exercises to stretch your muscles after an MVA. Ask your healthcare provider for exercises you can do.

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12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Coding Summary

Account Information

Hospital Account	Primary Payor	Affiliated Recurring Accounts	Combined from HAR
[redacted] - [redacted]	UNITED HEALTHCARE [201]	None	None

Admission Information

Arrival Date/Time:	12/02/2022 1444	Admit Date/Time:	12/02/2022 1444	IP Adm. Date/Time:	
Admission Type:	Emergency	Point of Origin:	Non-health Care Facility Point Of Origin	Admit Category:	
Means of Arrival:	Car	Primary Service:	Emergency	Secondary Service:	
Transfer Source:		Service Area:	BJC HEALTHCARE	Unit:	Christian Hospital Northwest Emergency Department
Admit Provider:		Attending Provider:	Abbott, Robert Emmons, MD	Referring Provider:	

Discharge Information

Discharge Date/Time	Discharge Disposition	Discharge Destination	Discharge Provider	Unit
12/02/2022 1712	Discharge To Home Or Self Care	None	None	Christian Hospital Northwest Emergency Department

Admission Diagnoses / Reasons for Visit (ICD-10-CM)

Code	Description	Comments
S39.012A	Strain of muscle, fascia and tendon of lower back, initial encounter	

Final Diagnoses (ICD-10-CM)

Code	Description	POA	CC	HAC	Affects DRG
S39.012A **[Principal]**	Strain of muscle, fascia and tendon of lower back, initial encounter				
S80.01XA	Contusion of right knee, initial encounter				
S29.012A	Strain of muscle and tendon of back wall of thorax, initial encounter				
Z86.16	Personal history of COVID-19				
Z83.3	Family history of diabetes mellitus				

External Causes of Injury (ICD-10-CM)

Code	Description	POA	CC	HAC	Affects DRG
V43.52XA	Car driver injured in collision with other type car in traffic accident, initial encounter				
Y92.410	Unspecified street and highway as the place of occurrence of the external cause				

BJC HealthCare

[redacted]

HAR: [redacted] MRN: [redacted]

[redacted] male

12/02/2022

Christian Hospital NorthWest HealthCare

CONSENT AND AUTHORIZATION

I know that I have the right to make decisions about my/my child's medical treatment. I consent to have the doctors and other healthcare workers at Christian Hospital NorthWest HealthCare ("Hospital") provide medical treatment to me/my child. I understand the medical treatment is provided by physicians and health care workers who may be employees of the Hospital, Washington University School of Medicine, other physician groups, or individuals who are allowed to provide care at the Hospital. I also understand the physicians and health care workers may be fellows, residents or students in training and that the individuals caring for me/my child are collectively called "the staff."

I consent to taking and storing photographs, videos and other electronic images of me/my child for the purposes of treating me/my child and providing education and I understand that reasonable efforts will be made to protect the identity of me/my child.

If I receive Medicare or Medicaid, I agree the information I gave to apply for payment is correct. I have been given a paper listing my rights as a Medicare or Medicaid patient. I know I can ask for a review of my/my child's record to find out about any payments or charges I may owe if Medicare or Medicaid will not cover my/my child's charges.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

If I receive Medicare, Medicaid or other insurance benefits, I know I am responsible to know what my insurance covers and that I can call my insurance plan if I have questions. I also understand I am responsible for any deductibles, co-insurance, and any non-covered charges. I know that I may receive separate bills for services provided by healthcare workers who are not employed by the Hospital – like Washington University School of Medicine, other physicians or other individuals authorized to provide care at the Hospital.

I authorize direct payment to the Hospital of all insurance benefits and I authorize release of my/my child's health information necessary for payment purposes. I understand that I am responsible, subject to BJC's Financial Assistance Policy, to pay for portions of my/my child's hospital bill not covered by insurance.

I also agree that I have received an Admission Packet, or Toolkit, if this information applies to me. If not, I have access to the information through signs and/or brochures which contain information about:

- Advance Directives: What are they? Where can I get one? Do we need one?
- Privacy of my health care information and who may have access to my information
- How the Hospital handles personal valuables
- Visiting Hours, Visitor Policies and Behavior Rules
- The Rights and Responsibilities I/we have as a patient or family member and who to contact if I/we have questions.

I have read this whole form, or had it read and explained to me, and I had the opportunity to ask questions.

Signature of Person Consenting to Treatment:

[redacted]

Signature captured with Topaz by [redacted] at 12/2/2022 03:13 PM

Relationship to Patient:

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

Signature of Guarantor if different than above:

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

BJC Accept Auth Unsecured Communications - Electronic signature on 12/2/2022 3:13 PM (effective from 12/2/2022) - E-signed

[redacted]

HAR: [redacted] MRN: [redacted]

[redacted] male

12/02/2022

AUTHORIZATION TO UTILIZE UNSECURE COMMUNICATIONS FOR SHARING PROTECTED HEALTH INFORMATION

Electronic mail (email) and text messaging are very common and convenient forms of communication. These methods may be used to share information between you and your health care providers. As a patient, you have the right to confidentiality of your Protected Health Information (PHI). It is important for you to understand that there is a potential risk that email and text messages containing your protected health information may be intercepted by a third party.

Before BJC HealthCare and its affiliated hospitals (each, a "BJC Hospital"), can send you email or text messages, we are required to make you aware of these risks and to obtain your authorization. **If you elect to receive communications in this manner, we will (when available to the applicable health care provider) use the information you provided to any BJC HealthCare affiliated provider or any Washington University School of Medicine provider to send phone messages, texts, and emails regarding your upcoming or missed appointments, to follow-up regarding your clinical management and experiences, and to inquire about symptoms and events as determined by each department and/or entity.** We will limit information sent to you to the minimum deemed necessary to benefit your health.

Email, phone, text and other messaging communications may become a part of your patient medical record and may be accessible to the clinical support staff responsible for your care.

Your email, phone, and text responses will be recorded and reviewed by a designated healthcare provider. Not all responses will receive follow-up. If you expect a response from your provider's office and have not heard back, please call the office during regular business hours.

EMAIL AND TEXT MESSAGE COMMUNICATION SHOULD NEVER BE USED IN THE CASE OF AN EMERGENCY OR FOR URGENT REQUESTS FOR INFORMATION.

By supplying my home phone number, mobile phone number, email address, and any other personal contact information, I authorize BJC HealthCare and BJC Hospitals to employ a third-party automated outreach and messaging system to use my personal information, the name of my care provider, the time and place of my scheduled appointment(s), and other limited information, for the purpose of notifying me of a pending or missed appointment. I understand that notifications may use an autodialer and/or prerecorded or artificial voice and may be repeated multiple times per appointment and may exceed contacts more than three times per week. I also authorize my

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

healthcare provider to disclose to third parties, who may intercept these messages, notification of a pending or missed appointment.

If you elect to communicate from a workplace computer, email account, or workplace phone, please be aware that your employer and workplace agents might have access to those email and text communications.

This authorization may be revoked at any time and must be done in writing. It is understood that the revocation will not apply to information that has already been released.
I agree to update you if my phone number or email address change. I understand this authorization is valid while in a treatment relationship with any BJC Hospital.

My signature indicates that I understand and accept the terms and conditions outlined herein.

I AUTHORIZE BJC HealthCare and BJC Hospitals to send me unsecured email and text messages to the phone number and email address I provide to any BJC HealthCare affiliated provider or any Washington University School of Medicine health care provider. I understand that changes to my MyChart Account will also impact where such email and text messages are delivered.

Authorize Unsecure Communications

[redacted]

Signature captured with Topaz by [redacted] at 12/2/2022 03:13 PM

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

After Visit Summary - Document on 12/2/2022 5:00 PM: Stop the Spread of Germs

Document (below)

[redacted]#[redacted](Acct:[redacted]) ([redacted] y.o. M) (Adm: 12/02/22) PCP: NO, P (999-999-9999) PED24 (In Process)

Stop the Spread of Germs

Stop the Spread of Ger

Help prevent the spread of respiratory diseases like COVID-19.

cdc.gov/coronavirus

[redacted] (MR # [redacted]) Printed at 12/2/2022 5:00 PM Page 1 of 1

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

After Visit Summary - Document on 12/2/2022 5:00 PM: ED After Visit Summary

Document (below)

AFTER VISIT SUMMARY

BJC HealthCare

[redacted] DoB [redacted]

12/2/2022 Christian Hospital Northwest Emergency Department 314-953-6994

Instructions

Take the medications as prescribed. Follow up with your doctor or the doctor provided for any new or worsening concerns.

Today's Visit

You were seen by Robert Emmons Abbott, MD and Kimberly Janean King, NP

Reason for Visit
Motor Vehicle Crash

Diagnoses
- Motor vehicle accident, initial encounter
- Strain of trapezius muscle, unspecified laterality, initial encounter
- Strain of lumbar region, initial encounter
- Contusion of right knee, initial encounter

Your medication changes

START taking
methocarbamol (ROBAXIN)

naproxen (NAPROSYN)

CHANGE how you take
acetaminophen (TYLENOL) medication strength, how much to take, reasons to take this

Review your updated medication list below.

Your medication information

1. Contusion in Adults (AfterCare(R) Instructions(ER/ED)) (English)
2. Muscle Strain (AfterCare(R) Instructions(ER/ED)) (English)
3. Motor Vehicle Accident (AfterCare(R) Instructions(ER/ED)) (English)

[illegible]

acetaminophen • methocarbamol • naproxen

Address: 12066 JUSTUS RD SAINT LOUIS MO 63138-1362
Phone: 314-355-6500

[illegible]

Why: As needed, If symptoms worsen

[illegible]

Why: As needed, If symptoms worsen, To establish primary care
Specialty: Interventional Cardiology, Internal Medicine
Contact: 637 DUNN RD STE 170
Hazelwood MO 63042
314 838 5702

Imaging Tests
XR Knee Right 4 or More Views
XR Spine Lumbar 4 or More Views

Medications Given
acetaminophen (TYLENOL) tablet 650 mg Last given at 3:58 PM
ibuprofen (ADVIL,MOTRIN) tablet 800 mg Last given at 3:58 PM

MyChart

View your After Visit Summary and more online at https://www.mypatientchart.org/MyChart/.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

What's Next

You currently have no upcoming appointments scheduled.

Your Medication List

TAKE these medications

acetaminophen 500 mg tablet
TYLENOL

Take 1 tablet (500 mg total) by mouth every 6 (six) hours as needed for headaches or pain

- **medication strength**
- **how much to take**
- **reasons to take this**

START

methocarbamol 500 mg tablet
ROBAXIN

Take 1 tablet (500 mg total) by mouth 2 (two) times a day

START

naproxen 500 mg tablet
NAPROSYN

Take 1 tablet (500 mg total) by mouth 2 (two) times a day with meals

ASK your doctor about these medications

ASK

bisacodyl EC 5 mg EC tablet
DULCOLAX EC

Take 2 tablets (10 mg total) by mouth daily as needed for constipation (If no results 24 hours after polyethylene glycol (MIRALAX). May give bisacodyl supp if not tolerating PO))

ASK

famotidine 20 mg tablet
PEPCID

Take 1 tablet (20 mg total) by mouth 2 (two) times a day

ASK

montelukast 10 mg tablet
SINGULAIR

Take 1 tablet (10 mg total) by mouth nightly

ASK

ondansetron 4 mg tablet
ZOFRAN

Take 1 tablet (4 mg total) by mouth every 8 (eight) hours as needed for nausea or vomiting

ASK

polyethylene glycol 17 gram packet
MIRALAX

Take 1 packet (17 g total) by mouth daily as needed for constipation

Physician Referral Line

If you currently do not have a primary care physician, please call the physician referral line at: **314-747-WELL or 1-877-747-WELL**.

The Emergency Department is for the treatment of medical/surgical emergencies. It is very important that you be seen by your primary care physician or home clinic for on-going care within the time frame recommended by your ED provider.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

The emergency physician may have provided a preliminary interpretation of your x-rays and/or EKG. If so, a specialist will perform a final interpretation of these tests. If a change in your diagnosis or treatment is needed, we will contact you. It is critical that we have a current phone number for you.

If you experience any thoughts of harming yourself, contact the **National Suicide Prevention Lifeline** at **988**.

Resources available for youth experiencing any thoughts of self-harm include:
Behavioral Health Response Missouri Crisis Hotlines: **314-469-6644** (local), **1-800-811-4760** (toll free)
Youth in Crisis at Behavioral Health Response: **1-844-985-8282** or Text: **BHEARD** to **31658** (message and data rates apply)

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

Attached Information — Contusion in Adults (AfterCare/R) Instructions,FR/ED): (English)

Contusion in Adults

WHAT YOU NEED TO KNOW:

A contusion is a bruise that appears on your skin after an injury. A bruise happens when small blood vessels tear but skin does not. When blood vessels tear, blood leaks into nearby tissue, such as soft tissue or muscle.

DISCHARGE INSTRUCTIONS:

Return to the emergency department if:

- You have new trouble moving the injured area.
- You have tingling or numbness in or near the injured area.
- Your hand or foot below the bruise gets cold or turns pale.

Contact your healthcare provider if:

- You find a new lump in the injured area.
- Your symptoms do not improve with treatment after 4 to 5 days.
- You have questions or concerns about your condition or care.

Medicines: You **may** need any of the following:

- **NSAIDs** help decrease swelling and pain or fever. This medicine is available with or without a doctor's order. NSAIDs can cause stomach bleeding or kidney problems in certain people. If you take blood thinner medicine, **always** ask your healthcare provider if NSAIDs are safe for you. Always read the medicine label and follow directions.
- **Prescription pain medicine** may be given. Do not wait until the pain is severe before you take your medicine.
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell him of her if you are allergic to any medicine. Keep a list of the medicines, vitamins, and herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Follow up with your healthcare provider as directed: You may need to return within a week to check your injury again. Write down your questions so you remember to ask them during your visits.

Help a contusion heal:

- **Rest the injured area** or use it less than usual. If you bruised your leg or foot, you may need crutches or a cane to help you walk. This will help you keep weight off your injured body part.
- **Apply ice** to decrease swelling and pain. Ice may also help prevent tissue damage. Use an ice pack, or put crushed ice in a plastic bag. Cover it with a towel and place it on your bruise for 15 to 20 minutes every hour or as directed.

12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Documents (continued)

- **Use compression** to support the area and decrease swelling. Wrap an elastic bandage around the area over the bruised muscle. Make sure the bandage is not too tight. You should be able to fit 1 finger between the bandage and your skin.

- **Elevate (raise) your injured body part** above the level of your heart to help decrease pain and swelling. Use pillows, blankets, or rolled towels to elevate the area as often as you can.

- **Do not drink alcohol** as directed. Alcohol may slow healing.

- **Do not stretch injured muscles** right after your injury. Ask your healthcare provider when and how you may safely stretch after your injury. Gentle stretches can help increase your flexibility.

- **Do not massage the area or put heating pads** on the bruise right after your injury. Heat and massage may slow healing. Your healthcare provider may tell you to apply heat after several days. At that time, heat will start to help the injury heal.

Prevent another contusion:

- Stretch and warm up before you play sports or exercise.

- Wear protective gear when you play sports. Examples are shin guards and padding.

- If you begin a new physical activity, start slowly to give your body a chance to adjust.

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12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

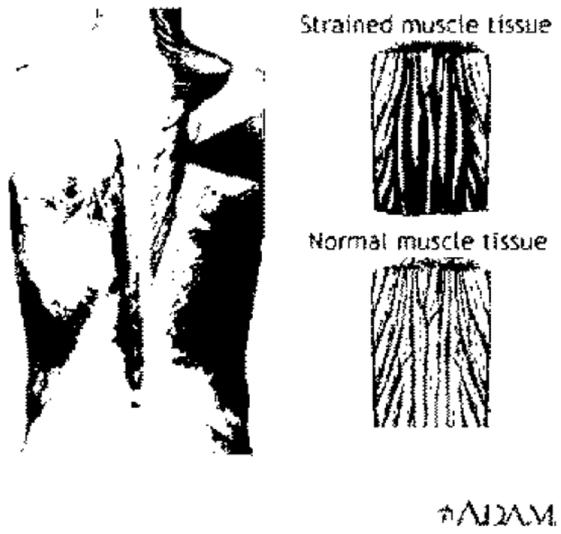
Documents (continued)

Attached Information — Muscle Strain (AfterCare(R) Instructions,ER/ED): (English)

Muscle Strain

WHAT YOU NEED TO KNOW:

A muscle strain is a twist, pull, or tear of a muscle or tendon. A tendon is a strong elastic tissue that connects a muscle to a bone. Signs of a strained muscle include bruising and swelling over the area, pain with movement, and loss of strength.



DISCHARGE INSTRUCTIONS:

Return to the emergency department if:

- You suddenly cannot feel or move your injured muscle.

Contact your healthcare provider if:

- Your pain and swelling worsen or do not go away.
- You have questions or concerns about your condition or care.

Medicines:

- **NSAIDs** help decrease swelling and pain or fever. This medicine is available with or without a doctor's order. NSAIDs can cause stomach bleeding or kidney problems in certain people. If you take blood thinner medicine, **always** ask your healthcare provider if NSAIDs are safe for you. Always read the medicine label and follow directions.
- **Muscle relaxers** help decrease pain and muscle spasms.
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell him of her if you are allergic to any medicine. Keep a list of the medicines, vitamins, and

herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Follow up with your healthcare provider as directed: Your healthcare provider may suggest that you have a follow up visit before you go back to your usual activity. Write down your questions so you remember to ask them during your visits.

Self-care:

- **3 to 7 days after the injury:** Use Rest, Ice, Compression, and Elevation (RICE) to help stop bruising and decrease pain and swelling.
    - **Rest:** Rest your muscle to allow your injury to heal. When the pain decreases, begin normal, slow movements. For mild and moderate muscle strains, you should rest your muscles for about 2 days. However, if you have a severe muscle strain, you should rest for 10 to 14 days. You may need to use crutches to walk if your muscle strain is in your legs or lower body.
    - **Ice:** Put an ice pack on the injured area. Put a towel between the ice pack and your skin. Do **not** put the ice pack directly on your skin. You can use a package of frozen peas instead of an ice pack.
    - **Compression:** You may need to wrap an elastic bandage around the area to decrease swelling. It should be tight enough for you to feel support. Do **not** wrap it too tightly.
    - **Elevation:** Keep the injured muscle raised above your heart if possible. For example if you have a strain of your lower leg muscle, lie down and prop your leg up on pillows. This helps decrease pain and swelling.
- **3 to 21 days after the injury:** Start to slowly and regularly exercise your muscle. This will help it heal. If you feel pain, decrease how hard you are exercising.
- **1 to 6 weeks after the injury:** Stretch the injured muscle. Hold the stretch for about 30 seconds. Do this 4 times a day. You may stretch the muscle until you feel a slight pull. Stop stretching if you feel pain.
- **2 weeks to 6 months after the injury:** The goal of this phase is to return to the activity you were doing before the injury happened, without hurting the muscle again.
- **3 weeks to 6 months after the injury:** Keep stretching and strengthening your muscles to avoid injury. Slowly increase the time and distance that you exercise. You may have signs and symptoms of muscle strain 6 months after the injury, even if you do things to help it heal. In this case, you may need surgery on the muscle.

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Attached Information — Motor Vehicle Accident (AfterCare/R) Instructions.FR/ED): (English)

Motor Vehicle Accident

WHAT YOU NEED TO KNOW:

A motor vehicle accident (MVA) can cause injury from the impact or from being thrown around inside the car. You may have a bruise on your abdomen, chest, or neck from the seatbelt. You may also have pain in your face, neck, or back. You may have pain in your knee, hip, or thigh if your body hits the dash or the steering wheel. Muscle pain is commonly worse 1 to 2 days after an MVA.

DISCHARGE INSTRUCTIONS:

Call 911 if:

- You have new or worsening chest pain or shortness of breath.

Return to the emergency department if:

- You have new or worsening pain in your abdomen.
- You have nausea and vomiting that does not get better.
- You have a severe headache.
- You have weakness, tingling, or numbness in your arms or legs.
- You have new or worsening pain that makes it hard for you to move.

Contact your healthcare provider if:

- You have pain that develops 2 to 3 days after the MVA.
- You have questions or concerns about your condition or care.

Medicines:

- **Pain medicine:** You may be given medicine to take away or decrease pain. Do not wait until the pain is severe before you take your medicine.
- **NSAIDs**, such as ibuprofen, help decrease swelling, pain, and fever. This medicine is available with or without a doctor's order. NSAIDs can cause stomach bleeding or kidney problems in certain people. If you take blood thinner medicine, **always** ask if NSAIDs are safe for you. Always read the medicine label and follow directions. **Do not give these medicines to children under 6 months of age without direction from your child's healthcare provider.**
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell him of her if you are allergic to any medicine. Keep a list of the medicines, vitamins, and herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Follow up with your healthcare provider as directed: Write down your questions so you remember to ask them during your visits.

Safety tips:

- **Always wear your seatbelt.** This will help reduce serious injury from an MVA.
- **Use child safety seats.** Your child needs to ride in a child safety seat made for his age, height, and weight. Ask your healthcare provider for more information about child safety seats.
- **Decrease speed.** Drive the speed limit to reduce your risk for an MVA.
- **Do not drive if you are tired.** You will react more slowly when you are tired. The slowed reaction time will increase your risk for an MVA.
- **Do not talk or text on your cell phone while you drive.** You cannot respond fast enough in an emergency if you are distracted by texts or conversations.
- **Do not drink and drive.** Use a designated driver. Call a taxi or get a ride home with someone if you have been drinking. Do not let your friends drive if they have been drinking alcohol.
- **Do not use illegal drugs and drive.** You may be more tired or take risks that you normally would not take. Do not drive after you take prescription medicines that make you sleepy.

Self-care:

- **Use ice and heat.** Ice helps decrease swelling and pain. Ice may also help prevent tissue damage. Use an ice pack, or put crushed ice in a plastic bag. Cover it with a towel and apply to your injured area for 15 to 20 minutes every hour, or as directed. After 2 days, use a heating pad on your injured area. Use heat as directed.
- **Gently stretch.** Use gentle exercises to stretch your muscles after an MVA. Ask your healthcare provider for exercises you can do.

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12/02/2022 - ED in Christian Hospital Northwest Emergency Department (continued)

Flowsheets (all recorded)

Newborn Hearing Screen

No documentation.

Itemized Billing Statement

Date Range: Dec 13, 2022 Mar 21, 2023

Provider: All **Patient:** [redacted]

PATIENT NAME: [redacted] **ACCOUNT #:** [redacted] **DOB:** [redacted]

MOBILE PHONE: **ADDRESS:** [redacted] **PHONE:** [redacted]

FacilityName	Rendering Provider Name	Case ID	CLAIM #	Service Date	CODE/DESC	Mod1	Units	ICD	BALANCE
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	20552 INJECT TRIGGER POINT, 1 OR 2		1	M54 2 M79.18 M54.50 M25.561	$316.90
Ortho Sport and Spine Physicians St Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	72040 X RAY EXAM OF NECK SPINE		1	M54.2 M79.18 M54 50 M25.561	$133.95
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	72100 X-RAY EXAM OF LOWER SPINE		1	M54 2 M79.18 M54.50 M25.561	$133.95
Ortho Sport and Spine Physicians St Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	73562 X RAY EXAM OF KNEE, 3	RT	1	M54.2 M79.18 M54 50 M25.561	$133.95
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	99070 CYCLOBENZAPRINE 10MG #30		1	M54 2 M79.18 M54.50 M25.561	$35.65
Ortho Sport and Spine Physicians St Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	99070 DENDRACIN 30/10/0.25%		1	M54.2 M79.18 M54 50 M25.561	$505.73
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	99072 PPE Related to Public Health Emergency		1	M54 2 M79.18 M54.50 M25.561	$0.00
Ortho Sport and Spine Physicians St Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	99204 Office Visit New Pt., Level 4	25	1	M54.2 M79.18 M54 50 M25.561	$543.41
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	E0730 Tens four lead	NU	1	M54 2 M79.18 M54.50 M25.561	$695.00
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	J3301 INJ TRIAMCINOLONE ACETONIDE 10 MG		1	M54.2 M79.18 M54 50	$69.00

								M25.561	
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	J3490 INJECTABLE, MARCAINE .25% 1CC		1	M54.2 M79.18 M54.50 M25.561	$38.00
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	L0637 LSO SAG-CORONAL PANEL PREFAB	NU	1	M54.2 M79.18 M54.50 M25.561	$1,742.40
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	416293	Dec 13, 2022	L1820 KNEE Hinged Brace	NU	1	M54.2 M79.18 M54.50 M25.561	$859.22
CLAIM BALANCE									**$5,207.16**
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	20552 INJECT TRIGGER POINT, 1 OR 2		1	M54.50 M54.2 M79.18	$342.25
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	99070 CYCLOBENZAPRINE 10MG #30		1	M54.50 M54.2 M79.18	$35.65
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	99070 DENDRACIN 30/10/0.25%		1	M54.50 M54.2 M79.18	$505.73
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	99072 PPE Related to Public Health Emergency		1	M54.50 M54.2 M79.18	$0.00
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	99214 Office Visit, Est Pt., Level 4	25	1	M54.50 M54.2 M79.18	$382.24
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	E0730 Tens four lead	NU	1	M54.50 M54.2 M79.18	$750.60
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	J2001 INJECTION, XYLOCAINE 1% 1CC		1	M54.50 M54.2 M79.18	$70.40
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	J3301 INJ TRIAMCINOLONE ACETONIDE 10 MG		1	M54.50 M54.2 M79.18	$69.00
Ortho Sport and Spine Physicians St. Louis	Lane, Nicole	61,922	428264	Feb 7, 2023	T2002 #3 Non-emergency transportation; per diem		1	M54.50 M54.2 M79.18	$450.00
CLAIM									**$2,605.87**

BALANCE	
ACCOUNT SUMMARY	**$7,813.03**

Mar 21, 2023 1 3:05:08 PM

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** [redacted]

Patient: [redacted]
Account Number: [redacted]
DOB: 04/23/1982 **Age:** [redacted]Y **Sex:** Male
Phone: [redacted]
Address: [redacted]

Provider: Nicole Lane, NP-C
Date: 12/13/2022

Subjective:

Chief Complaints:

1. Patient presents today complaining of neck, low back and right knee pain.

HPI:

Constitutional:

This is a [redacted]-year-old gentleman who presents today with complaints of neck pain, low back pain, and right knee pain. He was involved in a motor vehicle accident on 12/1/2022. He was a restrained driver and briefly lost consciousness. Shortly after the accident, he was evaluated in the emergency room at Christian Northeast Hospital where he was treated with x-rays and medications and later released. He denies having any pain prior to the accident. He now complains of severe neck, low back pain and right knee pain.

In regards to his neck pain, he describes it as an aching tightness that starts posteriorly and radiates down his bilateral shoulders and bilateral scapular region, left side worse than right. He is having muscle spasms on a daily basis. He is having a new onset of headaches. He denies any numbness, tingling, or weakness in his upper extremities.

In regards to his low back, he states it is an aching pain across his low back that worsens with increased physical activity. He has difficulty with lifting, standing, bending, and sitting for prolonged periods of time. He is having difficulty sleeping at night.

He also complains of right knee pain. He states the pain is along his patellar tendon and medial and lateral joint lines. He states having pain with going up stairs squatting, or lifting. He describes it as a sharp shooting pain through his knee that makes his knee feel like it can buckle at times.

He has not had any MRI imaging, physical therapy/chiropractic care. He has not had any injections at this point. He currently rates his right knee pain a 5/10 on a verbal pain scale. He rates his neck and low back pain a 7/10 on the pain scale.

ROS:

General/Constitutional:

Unexplained weight loss **admits**. Change in appetite **admits**. Fever or chills Denies. Night sweats Denies. Marked fatigue Denies. Diffuculty sleeping **admits**.

Ophthalmologic:

Glasses Denies. Change of vision **admits**.

ENT:

Decreased hearing Denies. Difficulty swallowing Denies. Ear pain Denies. Hoarseness Denies. Nosebleed Denies.

Respiratory:

Morning cough Denies. Shortness of breath Denies. Productive cough or sputum Denies.

Gastrointestinal:

Stomach pain / ulcers **admits**. Constipation **admits**. Diarrhea Denies. Nausea Denies. Vomiting Denies. Blood in stool **admits**.

Genitourinary:

Burning with urination Denies. Difficulty starting urination Denies. Pelvic pain Denies. Unable to empty bladder Denies.

Musculoskeletal:

Neck pain **admits**. Back pain **admits**. Muscle aches **admits**. Painful joints **admits**.

Skin:

Rash Denies. Easy bruising Denies. Swollen ankles Denies.

Neurologic:

Fainting Denies. Headache **admits**. Seizures Denies. Tremor Denies.

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** 12/13/2022

Medical History: Ulcerative colitis, Gastritis.

Surgical History: Boxer Fracture- plates and pins in hand 2017.

Hospitalization/Major Diagnostic Procedure: Denies Past Hospitalization.

Family History: No Family History documented..

Social History:

Tobacco Use:
Tobacco Use/Smoking
Are you a *Current smoker*

Hand dominance:
Hand Dominance: I am Right Handed.

Drugs/Alcohol:
Alcohol Screen (Audit-C)
Did you have a drink containing alcohol in the past year? *No*

Miscellaneous:
Occupation: Painter.

Medications: None

Allergies: Rye Bread.

Implants:

Objective:

Vitals: Ht **6 ft 2 in**, Wt **170 lbs**, BMI **21.82 Index**, BP **116/62 mm Hg**, HR **69 /min**, Pain scale **7 1-10**, Ht-cm **187.96**, Wt-kg **77.11**.

Examination:

General Examination:

The patient is alert and oriented x3, no apparent distress, chest with normal respiratory effort, regular rate and non- labored breathing. Abdomen soft, non-tender. PERRLA, skin intact without erythema, rash. Adequately aligned spine. No joint erythema or tenderness. Normal muscular development.

CERVICAL:l

Normal cervical lordosis is maintained. Patient demonstrates limited range of motion on extension and flexion. There is pain present with flexion and extension as well as lateral rotation bilaterally. There is tenderness along the paraspinal muscles bilaterally, left worse than right side. There is a small palpable trigger point noted along his left scapular region. Strength motor exam is 5/5 in all muscle groups, deep tendon reflexes are 2/4 bilaterally at brachioradialis and biceps reflexes. Spurling sign positive bilaterally, clonus negative bilaterally, Phalens negative bilaterally.

LUMBAR:

Normal lumbar lordosis is maintained. Patient demonstrates limited range of motion with flexion and extension. There is pain in flexion. No obvious pelvic obliquity. No tenderness on the lumbar spinal processes. There is tenderness on palpation to bilateral paraspinal muscles. Positive facet loading sign with flexion. Strength is 5/5 bilaterally in all muscle groups. Deep tendon reflexes is 2/4 with bilateral patellar and achilles. There is tenderness on palpation over the bilateral SI joint. Straight leg raise negative bilaterally. Faber exam negative bilaterally. Thigh thrust and compression exam negative bilaterally.

RIGHT KNEE:

The patient has a normal gait pattern. There is 1+ effusion present to the anterior aspect of the patients right knee. There is no erythema, ecchymosis present. Bench testing of the patients right knee reveals 5 degrees of extension and 115 degrees of flexion. There is tenderness on palpation to the medial and lateral joint lines. The patient is stable to valgus stress. There is slight laxity with varus stress to the right knee. ACL instability- Lachman exam negative, PCL instability- Posterior drawer test negative, Meniscus tear- McMurray exam not tested due to patient's complaint of severe pain. The patient has patent pulses 2+ bilaterally. Motors and sensory are intact bilaterally.

Assessment:

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** 12/13/2022

Assessment:

1. Cervicalgia - M54.2 (Primary)
2. Myofascial muscle pain - M79.18
3. Trigger point of neck - M54.2
4. Lumbago - M54.50
5. Right knee pain - M25.561

Plan:

1. Cervicalgia

Imaging: Xray Cervical spine 2 View
Dunville, Morgan 12/14/2022 11:25:31 AM >
Imaging: MRI Cervical w/o Contrast
Dunville, Morgan 12/14/2022 11:26:22 AM >

2. Lumbago

Start Dendracin Neurodendraxcin Lotion, 0.025-10-30 %, Apply as needed to affected area, Externally, 2-3 times a day, 30 days, 1 Tube, Refills 0 ; Start Cyclobenzaprine HCl Tablet, 10 MG, 1 tablet as needed, Orally, Once a day at bedtime, 30 days, Refills 0 ; Start Back Support L/XL Miscellaneous, -, as directed ; Start TENS Therapy Pain Relief Device, -, as directed, as needed .

Imaging: Xray Lumbar Spine 2 View
Dunville, Morgan 12/14/2022 11:26:37 AM >
Imaging: MRI Lumbar w/o Contrast
Dunville, Morgan 12/14/2022 11:26:52 AM >

3. Right knee pain

Start Knee Brace/Hinged Bars Medium Miscellaneous, -, as directed .

Imaging: Xray Right Knee 3 View
Dunville, Morgan 12/14/2022 11:27:09 AM >
Imaging: MRI Right Knee wo
Dunville, Morgan 12/14/2022 11:27:24 AM >

4. Others

Notes: **X-ray**:
X-ray of the cervical spine x2 views taken in office today shows straightening of the cervical lordosis, likely paraspinal muscle spasm. Disc heights are well-maintained without evidence of osteophyte formation, spondylolysis. There is no acute fracture present.
X-ray of the lumbar spine x2 views taken in office today shows straightening of lordotic alignment, likely paraspinal muscle spasm. There is severe disc narrowing at T10-11, T11-T12, T12-L1, L5-S1 most severe at L5-S1. There is no evidence of any acute fracture.
X-ray of the right knee x3 views taken today in office show overall good alignment. There is no evidence of acute fracture. There is some mild narrowing in the medial joint line and the patellar femoral joint. There is no osteophyte formation. The patella is slightly lateralized in alignment on the sunrise view.

PLAN:
1. The patient is symptomatic for cervical and lumbar facet mediated pain. I ordered a cervical and lumbar MRI in order to review the results and determine the next steps to the treatment plan.
2. On the patient cervical exam, I noted a small palpable trigger point along his left scapular region. The patient was very tender on palpation. I offered the patient a left-sided trigger point injection today in office. The risk and benefits of this procedure were discussed with him today. He would like to proceed with this.
3. I provided the patient with a LSO brace for stability and during physical activity. I provided the patient with a TENs unit for myofascial pain. I prescribed Flexeril and Dendracin cream for pain relief. This was all dispensed in the office today. The patient was instructed to take the medication as prescribed.
4. In regards to the right knee, I feel the patient may have an MCL sprain and right knee joint effusion. I ordered a right knee MRI to rule out any soft tissue injury. I provided the patient with a hinged knee brace to help with stability during physical activity.
5. I will see the patient in 2-3 weeks for MRI review and determine future treatment options.

Provider: Nicole Lane, NP-C **Date: 12/13/2022**

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** 12/13/2022

Dictated by Nicole Lane, APRN, NP-C for Dr Jeffrey Lee, DO.

Procedures:

Risks and benefits of the joint injection were discussed with the patient today and the patient agreed to proceed. The patient was placed on the exam table in a seated position on the exam chair. The patients area of maximal tenderness was identified and marked. The area over the myofascial spasm was prepped with betadine and alcohol using sterile technique. After isolating it between 2 palpating fingertips, a 25 gauge 1.5 inch needle was placed in the center of the spasm. A negative aspiration was performed. Using a fanning technique, dry needling was performed. Next, a combination of 0.25% bupivacaine (1ml) and 40mg/ml Kenalog (0.5ml) was injected without difficulty. A bandaid was placed on the injection site. The patient tolerated the procedure well. The patient was instructed to ice the affected area and rest today, resuming normal activity tomorrow as tolerated.

Procedure Codes: 72040 X-RAY EXAM OF NECK SPINE, 72100 X-RAY EXAM OF LOWER SPINE, 73562 X-RAY EXAM OF KNEE, 3, 20552 INJECT TRIGGER POINT, 1 OR 2, J3490 INJECTABLE, MARCAINE .25% 1CC, J3301 Triamcinalone (Kenalog) Inj 40 MG / CC, 99072 PPE Related to Public Health Emergency, L1820 KNEE Hinged Brace, L0637 LSO SAG-CORONAL PANEL PREFAB, E0730 Tens four lead, 99070 DENDRACIN 30/10/0.25%, 99070 CYCLOBENZAPRINE 10MG #30

Follow Up: 3W

Electronically signed by Nicole Lane , APRN on 12/15/2022 at 10:37 AM EST
Sign off status: Completed

Provider: Nicole Lane, NP-C **Date: 12/13/2022**

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** 02/07/2023

Patient: [redacted]
Account Number: [redacted]
DOB: [redacted] **Age:** [redacted] Y **Sex:** Male
Phone: [redacted]
Address: [redacted]

Provider: Nicole Lane, NP-C
Date: 02/07/2023

Subjective:

Chief Complaints:

1. Patient returns today complaining of neck, low back, and right knee pain.

HPI:

Constitutional:

This is a [redacted]-year-old gentleman who presents today for follow-up of his neck, low back, and right knee pain. His date of injury was on 12/1/2022. He states having no relief since his last office visit. His main complaint is continued low back and neck pain. He describes the pain as an aching pain that worsens with physical activity. He is having difficulty with bending, twisting, and lifting. He is having difficulty making it through a workday without pain. He denies any numbness, tingling, or weakness in his upper or lower extremities today. He has not scheduled his MRI imaging yet. He has not had any physical therapy/chiropractic care. He currently rates his pain at 7-8/10 on a verbal pain scale.

ROS:

General/Constitutional:

Weight Gain denies. Unexplained weight loss denies. Change in appetite denies. Fever or chills denies. Night sweats denies. Marked fatigue denies. Difficulty sleeping denies.

Gastrointestinal:

Bowel / Bladder issues denies. Stomach pain / ulcers denies. Blood in stool denies. Constipation denies. Diarrhea denies. Hemorrhoids denies. Nausea denies. Vomiting denies.

Genitourinary:

Abdominal pain/swelling denies. Difficulty urinating denies. Burning with urination denies. Pelvic pain denies. Unable to empty bladder denies.

Musculoskeletal:

Neck pain **admits**. Back pain **admits**. Muscle aches denies. Painful joints **admits**.

Neurologic:

Balance difficulty denies. Fainting denies. Headache denies. Seizures denies. Tingling/Numbness denies. Blurred Vision denies. Tremor denies.

Medical History: Ulcerative colitis, Gastritis.

Surgical History: Boxer Fracture- plates and pins in hand 2017.

Hospitalization/Major Diagnostic Procedure: Denies Past Hospitalization.

Family History: No Family History documented..

Social History:

Tobacco Use:

Tobacco Use/Smoking

Are you a *Current smoker*

Hand dominance:

Hand Dominance: I am Right Handed.

Drugs/Alcohol:

Alcohol Screen (Audit-C)

Did you have a drink containing alcohol in the past year? *No*

Miscellaneous:

Occupation: Painter.

Medications: Taking Back Support L/XL - Miscellaneous as directed , Taking Knee Brace/Hinged Bars Medium - Miscellaneous as directed , Not-Taking/PRN TENS Therapy Pain Relief - Device as directed , Medication List

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** 02/07/2023

reviewed and reconciled with the patient

Allergies: Rye Bread.

Implants: No known implant history.

Objective:

Vitals: Ht **6 ft 2 in**, Wt **170 lbs**, BMI **21.82 Index**, BP **121/78 mm Hg**, HR **65 /min**, Pain scale **2 1-10**, Ht-cm **187.96**, Wt-kg **77.11**.

Examination:

General Examination:

The patient is alert and oriented x3, no apparent distress, chest with normal respiratory effort, regular rate and non- labored breathing. Abdomen soft, non-tender. PERRLA, skin intact without erythema, rash. Adequately aligned spine. No joint erythema or tenderness. Normal muscular development.

CERVICAL:

Normal cervical lordosis is maintained. Patient demonstrates limited range of motion on extension and flexion. There is pain present with flexion and extension as well as lateral rotation bilaterally. There is tenderness along the paraspinal muscles bilaterally, left worse than right side. There is a small palpable trigger point noted along his bilateral scapular region, left worse than right side. Strength motor exam is 5/5 in all muscle groups, deep tendon reflexes are 2/4 bilaterally at brachioradialis and biceps reflexes. Spurling sign positive bilaterally, clonus negative bilaterally, Phalens negative bilaterally.

LUMBAR:

Normal lumbar lordosis is maintained. Patient demonstrates limited range of motion with flexion and extension. There is pain in flexion. No obvious pelvic obliquity. No tenderness on the lumbar spinal processes. There is tenderness on palpation to bilateral paraspinal muscles. Positive facet loading sign with flexion. Strength is 5/5 bilaterally in all muscle groups. Deep tendon reflexes is 2/4 with bilateral patellar and achilles. There is tenderness on palpation over the bilateral SI joint. Straight leg raise negative bilaterally. Faber exam negative bilaterally. Thigh thrust and compression exam negative bilaterally.

Assessment:

Assessment:

1. Lumbago - M54.50 (Primary)
2. Cervicalgia - M54.2
3. Myofascial muscle pain - M79.18

Plan:

1. Lumbago

Refill Dendracin Neurodendraxcin Lotion, 0.025-10-30 %, Apply as needed to affected area, Externally, 2-3 times a day, 30 days, Refills 0 ; Refill TENS Therapy Pain Relief Device, -, as directed .

Referral To: OPEN CHIRO/PT Physical Therapist

Reason:Trigger point bilateral scapular region- dry needling as needed for trigger points | Cervical and lumbar facet joint pain | Eval and Treat 1-2 times a week for 6 weeks

2. Cervicalgia

Referral To: OPEN CHIRO/PT Physical Therapist

Reason:Trigger point bilateral scapular region- dry needling as needed for trigger points | Cervical and lumbar facet joint pain | Eval and Treat 1-2 times a week for 6 weeks

3. Others

Notes: 1. The patient seems to be suffering from cervical and lumbar facet mediated pain with with bilateral trigger points along his scapular region. He has not scheduled his MRI imaging yet. He had difficulty obtaining the studies due to transportation issues and his work schedule. I ordered new MRI imaging to be sent to CDI imaging due to their more accommodating work hours. He understands he will need to obtain the MRI imaging so I am able to offer him a more definitive treatment plan.

2. I ordered physical therapy for him today. He will attend 1-2 times a week for the next 6 weeks. I would like

[redacted] **DOB:** [redacted] ([redacted] yo M) **Acc No.** [redacted] **DOS:** 02/07/2023

him to start at Cora physical therapy so they are able to perform dry needling technique during the physical therapy sessions.
3. He will continue with conservative care, including his LSO brace, medications, and TENS unit. I provided him with a new TENS unit today, Dendracin cream. These were dispensed in the office.
4. The patient has 2 palpable trigger points along the scapular region. I offered the patient trigger point injections to help alleviate his pain. The risk and benefits of this procedure were discussed with him today and he would like to proceed.
5. I will follow-up with the patient in approximately 3 to 4 weeks for MRI review and to verify his progress from physical therapy.

Procedures:

Risks and benefits of the joint injection were discussed with the patient today and the patient agreed to proceed. The patient was placed on the exam table in a seated position on the exam chair. The patients area of maximal tenderness was identified and marked on his left side. The area over the myofascial spasm was prepped with betadine and alcohol using sterile technique. After isolating it between 2 palpating fingertips, a 25 gauge 1.5 inch needle was placed in the center of the spasm. A negative aspiration was performed. Using a fanning technique, dry needling was performed. Next, a combination of 1% Xylocaine (2ml) and 40mg/ml Kenalog (0.5ml) was injected without difficulty. A bandaid was placed on the injection site. The patient tolerated the procedure well.

Next, the area of max tenderness was identified and marked on his right side. The same procedure and combination of medications was performed for his right side. The patient tolerated the procedure well. He was instructed to ice the affected area and rest today, resuming normal activity tomorrow as tolerated.

Dictated by Nicole Lane, APRN, NP-C for Dr Jeffrey Lee, DO.

Procedure Codes: T2002 #3 Non-emergency transportation; per diem, 20552 INJECT TRIGGER POINT, 1 OR 2, J2001 INJECTION, XYLOCAINE 1% 1CC, J3301 Triamcinalone (Kenalog) Inj 40 MG / CC, 99072 PPE Related to Public Health Emergency, E0730 Tens four lead, 99070 DENDRACIN 30/10/0.25%, 99070 CYCLOBENZAPRINE 10MG #30

Follow Up: 3W

Electronically signed by Nicole Lane , APRN on 02/07/2023 at 04:30 PM EST
Sign off status: Completed

Provider: Nicole Lane, NP-C **Date: 02/07/2023**

Make Checks Payable To:

St Lukes Center for Diagnostic Imaging
PO Box 88199
Milwaukee, WI 53288-8199
USA
STATEMENT

STATEMENT DATE	PAY THIS AMOUNT	ACCOUNT NBR
03/20/2023	$0.00	[redacted]
SHOW AMOUNT PAID HERE $		

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[redacted]

☐ Please check box if above address is incorrect or insurance information has changed and indicate change(s) on reverse side.

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USA

PLEASE DETACH AND RETURN TOP PORTION WITH YOUR PAYMENT

Date	Description Of Service	Amount	Insurance Balance	Patient Balance	Balance
02/12/23	**ENCOUNTER FOR [redacted] WITH GEORGES MD, ALEXANDRA**				
02/12/23	73721 - MRI JNT OF LWR EXTRE W/O DYE	$2,368.00	$2,368.00		
	ENCOUNTER TOTAL	**$2,368.00**	**$2,368.00**	**$0.00**	**$2,368.00**
02/12/23	**ENCOUNTER FOR [redacted] WITH YALAVARTHI, KISHAN**				
02/12/23	72141 - MRI CERVICAL SPINE W/O DYE	$2,290.00	$2,290.00		
02/12/23	G9557 - PQRS 406 No Thyroid Nodule Noted	$0.00			
	ENCOUNTER TOTAL	**$2,290.00**	**$2,290.00**	**$0.00**	**$2,290.00**
02/12/23	**ENCOUNTER FOR [redacted] WITH ZHOU MD, YIHUA**				
02/12/23	72148 - MRI LUMBAR SPINE W/O DYE	$2,473.00	$2,473.00		
	ENCOUNTER TOTAL	**$2,473.00**	**$2,473.00**	**$0.00**	**$2,473.00**

This balance is your responsibility. Payment in full is due upon receipt.

Account Number	Current	30 Days	60 Days	90 Days	120 Days	Total Account Balance
[redacted]	$7,131.00	$0.00	$0.00	$0.00	$0.00	$7,131.00

MESSAGE:

Please Pay This
AMOUNT >>>> $0.00

** PAYMENT DUE UPON RECEIPT *THANK YOU **
STATEMENT

St. Luke's RAYUS RADIOLOGY

St. Lukes Rayus Chesterfield
Suite 101 6 McBride & Sons Ctr Drive
Chesterfield,MO,63005
Phone: (636-519-7865) Fax: (636-519-7866)

To: Nicole Lane, N.P.
Ste. 100
12401 Olive Blvd.
Creve Coeur,MO,63141
Phone: (314-390-0135)
Fax: (000-000-0000)

Name: [redacted]
MRN: [redacted] **Referring MRN:** [redacted]
Phone: ***
DOB: [redacted] **Gender:** Male
Exam Date: 02/12/2023
Referring Phys.: Nicole Lane, N.P.

EXAM: MRI EXTREM LOWER JT W/O CON RIGHT

HISTORY: Knee pain

TECHNIQUE: Multiplanar, multisequence MRI of the right knee was performed without contrast on a 1.5 Tesla magnet. There is no comparison.

FINDINGS: Within the medial compartment, the meniscus and articular cartilage are normal. There is minimal increased T2 signal about the medial aspect of the medial femoral condyle without a fracture line.

Within the lateral compartment, there is no meniscal tear. The articular cartilage is normal.

Within the patellofemoral joint, the articular cartilage is normal.

The anterior and posterior cruciate ligaments are intact. The medial collateral and lateral collateral ligament complex are normal. The extensor mechanism is normal.

There is a small joint effusion. There is a trace Baker''s cyst. The marrow signal is normal.

IMPRESSION:
1. Minimal nonspecific marrow edema of the medial aspect of the medial femoral condyle, this may represent a small contusion. There is no fracture.
2. Small joint effusion of the right knee.

Interpreting Physician

Alex Georges, M.D.
Final Report Electronically Signed: 2/13/23
8:55 am CT

St. Luke's RAYUS RADIOLOGY

St. Lukes Rayus Chesterfield
Suite 101 6 McBride & Sons Ctr Drive
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Creve Coeur,MO,63141
Phone: (314-390-0135)
Fax: (000-000-0000)

Name: [redacted]
MRN: [redacted] **Referring MRN:** [redacted]
Phone: ***
DOB: [redacted] **Gender:** Male
Exam Date: 02/12/2023
Referring Phys.: Nicole Lane, N.P.

EXAMINATION: Magnetic resonance imaging (MRI) of the lumbar spine without contrast

HISTORY: Low back pain. Bilateral knee pain.

TECHNIQUE: MRI of the lumbar spine was performed without contrast according to standard protocol.

COMPARISON: No prior study is available for comparison at the time of this dictation.

FINDINGS:

There is a transitional lumbosacral segment with a presumed partially sacralized L5. If absolute confirmation of the patient''s spinal numbering is necessary from a clinical standpoint, radiographic views of the thoracic and lumbar spine may be obtained.

The alignment is normal. Vertebral bodies are normal in height without evidence of compression fractures. Marrow signal intensity is normal. The conus medullaris terminates at the level of T12/L1 and the distal spinal cord signal intensity is normal. The intervertebral discs are normal in height.

No soft tissue abnormality is identified.

L1-L2: There is no disc bulge. There is no central canal stenosis. There is no facet osteoarthritis. There is no neural foraminal stenosis.

L2-L3: There is no disc bulge. There is no central canal stenosis. There is no facet osteoarthritis. There is no neural foraminal stenosis.

L3-L4: There is no disc bulge. There is no central canal stenosis. There is no facet osteoarthritis. There is no neural foraminal stenosis.

L4-L5: There is mild diffuse disc bulge and a small central disc protrusion. There is no central canal stenosis. There is mild bilateral facet osteoarthritis. There is no neural foraminal stenosis.

L5-S1: There is no disc bulge. There is no central canal stenosis. There is mild bilateral facet osteoarthritis. There is no neural foraminal stenosis.

IMPRESSION:

1. Mild multilevel degenerative disc and joint disease at L4-L5 and L5-S1 as detailed above. No central canal or neuroforaminal stenosis.

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Interpreting Physician

Yihua Zhou, M.D.
Final Report Electronically Signed: 2/13/23
10:28 am CT

St. Luke's RAYUS RADIOLOGY

St. Lukes Rayus Chesterfield
Suite 101 6 McBride & Sons Ctr Drive
Chesterfield,MO,63005
Phone: (636-519-7865) Fax: (636-519-7866)

To: Nicole Lane, N.P.
Ste. 100
12401 Olive Blvd.
Creve Coeur,MO,63141
Phone: (314-390-0135)
Fax: (000-000-0000)

Name: [redacted]
MRN: [redacted] **Referring MRN:** [redacted]
Phone: ***
DOB: [redacted] **Gender:** Male
Exam Date: 02/12/2023
Referring Phys.: Nicole Lane, N.P.

MRI CERVICAL SPINE WITHOUT CONTRAST

HISTORY: Neck pain, motor vehicle injury.

COMPARISON: None.

TECHNIQUE: MRI imaging was performed of the cervical spine without contrast in the high-field 1.5 Tesla magnet using the standard protocol.

FINDINGS: There is straightening of the normal cervical spine lordosis, without acute fracture, or marrow edema. There is normal signal in the cervical spinal cord and craniocervical junction. The cerebellar tonsils are normally positioned, without Chiari malformation.

C2-3: Within normal limits.

C3-4: There is no disc bulging, herniation, canal, or foraminal stenosis. There is mild bilateral uncovertebral joint disease.

C4-5: There is disc bulging, without focal herniation, canal, or foraminal stenosis. There is mild bilateral facet disease.

C5-6: There is no disc bulging, herniation, canal, or foraminal stenosis. There is mild left facet disease.

C6-7: There is disc bulging, with a superimposed right central and subarticular 5 mm AP disc herniation with an associated annular tear with 6 mm of caudal migration. There is right ventral cord impingement, with moderate to severe central canal stenosis with the AP diameter of the canal at 7 mm. There is bilateral uncovertebral joint and facet disease, with mild left and moderate right foraminal stenosis.

C7-T1: There is no disc bulging, herniation, canal, or foraminal stenosis.

IMPRESSION:

C6-7 right central and subarticular disc herniation with caudal migration with right ventral cord impingement, and moderate to severe central canal stenosis at this level.

KDY:jbs

Interpreting Physician

Kishan D. Yalavarthi, M.D.
Final Report Electronically Signed: 2/13/23
9:14 am CT